Stephen J. Joncus
Oregon Bar No. 013072
JONCUS LAW P.C.
13203 SE 172nd Ave Ste 166 #344
Happy Valley, Oregon 97086
Telephone: (971) 236-1200
Facsimile: (971) 244-7997
steve@joncus.net

David J. Schexnaydre, T.A., *Pro Hac Vice Pending*
SCHEXNAYDRE LAW FIRM
Louisiana Bar Roll #: 21073
Mississippi Bar Roll #: 102557
Texas Bar Roll #24076142
2895 Highway 190 • Suite 212
Mandeville, Louisiana 70471
Telephone: (985) 292-2020
Facsimile: (985) 235-1089
david@schexnaydre.com

*Attorneys for Plaintiffs*

UNITED STATES DISTRICT COURT

DISTRICT OF OREGON

EUGENE DIVISION

| | |
|---|---|
| SARA BOYSEN, *et al,*<br><br>*Plaintiffs*,<br><br>v.<br><br>PEACEHEALTH, LIZ DUNNE, DOUG, KOEKKOEK, TODD SALNAS, PATRICK ALLEN, AND GOV. KATE BROWN<br><br>Defendants. | Case No.: 6:23-cv-1229<br><br>**COMPLAINT**<br><br>Demand for Jury Trial |

NOW INTO COURT, through undersigned counsel, come Plaintiffs, Sara Boysen, *et al*, (hereinafter "Plaintiffs"), who respectfully file this Complaint against Defendants, PeaceHealth; its policymakers, the CEO Liz Dunne, Chief Physician Executive (CPE) Doug Koekkoek, and Todd Salnas, Chief Executive of PeaceHealth Oregon Network, in their individual and representative capacities; and Patrick Allen, in his official and personal capacity as Director of Oregon Health Authority, and Governor Kate Brown, in her official and personal capacity as Governor of the State of Oregon, (hereinafter "Defendants"), presenting allegations and causes of action as follows.

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## PRELIMINARY STATEMENT

This is an action brought under the United States Constitution, 42 U.S.C. § 1983, 21 U.S.C. §360bbb-3, 42 USC 247d-6d, 45 CFR 46, 18 U.S.C. §242, ICCPR Treaty, and the common laws of the State of Oregon to hold accountable Governor Kate Brown, Oregon Health Authority Director Patrick Allen, PeaceHealth, a State Actor at all times pertinent herein, via its policymakers, the CEO Liz Dunne, Chief Physician Executive (CPE) Doug Koekkoek, and Todd Salnas, Chief Executive of PeaceHealth Oregon Network, for damages arising out of their unconstitutional, unlawful, malicious, unequal and contractually violative COVID-19 investigational drug mandate. Special laws apply to Defendants' vaccine mandates because the

FDA defines the drugs at issue as "investigational with no license for any indication." And even though Defendants' vaccine mandates were instituted during and in response to a pandemic emergency, as the U.S. Supreme Court noted since the beginning of the pandemic: **"even in a pandemic, the Constitution cannot be put away and forgotten."** *Roman Catholic Diocese of Brooklyn v. Cuomo*, 141 S.Ct. 63, 208 L.Ed.2d 206 (2020).

## I. Introduction

1. On June 21, 1788, the United States Constitution became the supreme law of the land, serving as a beacon of hope worldwide.

2. The hope our forefathers enshrined in the Constitution is that everyone is created equal before the law, regardless of race, religion, nationality, or socioeconomic status.

3. An individual's right to be treated equally before the law bestows a duty upon the government to ensure that its laws, regulations, ordinances, and customs neither conflict with nor cancel that right. However, history has shown that our government sometimes breaches its duties to the citizenry. The courts must be called upon to prevent future breaches from occurring and to issue remedies to those damaged due to those wrongs.

4. In 1972, the nation became aware of the human rights abuses by the Executive Branch of the United States government. Using federal funds and authority, medical researchers effectively denied African-American males treatment for syphilis for no other reason than to study how the disease progressed in human anatomy. One-hundred male participants were allowed to suffer until death, 40 of their wives contracted syphilis, and 19 of their children were born with congenital syphilis.

5. Hiding behind the belief that the benefit of the many justifies the suffering of the few, medical researchers chose to engage in horrific crimes against humanity to further their political agendas over the Constitutional rights of individuals under their authority.

6. Enraged upon the discovery of this news, Senator Edward Kennedy held live hearings in 1973 detailing medical research abuses by the government, pharmaceutical corporations, and healthcare professionals. The nation was stunned to learn that medical research abuses impacting millions over decades had gone under the radar without criminal prosecution.

7. Some of those research abuses included:

A. the US Navy sprayed the entire city of San Francisco with a bacterial agent to study biological warfare (Operation Sea-Spray), injuring many unsuspecting residents;

B. Chester M. Southam of Sloan-Kettering Institute injected live cancer cells into 300 healthy female prisoners without informing them or asking permission;

C. In the early 1960s, Saul Krugman of Willowbrook State School in Staten Island, New York, deliberately infected children with viral hepatitis by feeding them extracts made from infected feces;

D. In 1966, the U.S. Army injected gas infused with bacteria throughout the New York City Subway system to study the impact of biological warfare;

E. In the 1950s, the Atomic Energy Commission (AEC) and Nebraska College of Medicine subjected healthy infants to radioactive iodine to test its effects on the thyroid gland;

F. Throughout the 1960s, Inuit natives in Alaska were treated with radioactive iodine without being informed of the potential dangers, nor did the AEC conduct any long-term follow-up.

G. The Department of Defense, in the late 1960s, funded non-consensual whole body radiation experiments on African-American, poor, and terminally ill persons, without informing them of the life-altering dangers.

H. Other illegal research activities numbering in the thousands included irradiating thousands of male testicles, removing skull parts of babies still in the womb, sterilizing black females, chemical baths, irradiating entire

towns with nuclear material, and injection of live cancer cells into prisoners and terminally ill patients.

8. Senator Kennedy's heroic effort to shut down entire industries using humans as fodder resulted in Congress passing the National Research Act in 1974. The Act laid the foundation for many laws, regulations, and ordinances to protect individuals regarding investigational medical products.

9. In the early 1980s, Congress established "the Common Rule" (45 CFR 46) as required compliance by federal agencies, departments, and the military when involving humans with investigational medical products.

10. The Common Rule was explicit in that no individual can be under "coercion," "undue influence," "unjustifiable pressure," or a "sanction" to participate in the use of medical products classified by the FDA as experimental. (45 CFR § 46.116, the Belmont Report)

11. In 2005, Congress enacted 21 U.S.C. §360bbb-3 (Authorization for Medical Products for Use in Emergencies), providing individuals with legal authority to participate in the use of medical products classified by the FDA as investigational when the HHS Secretary declared an emergency.

12. Acutely aware of its obligation to protect humans involved in the use of experimental medical products even during an emergency, Congress established "required conditions" consisting of two rights of the people and, respectively, two duties upon the government. One right grants individuals access to medical products authorized for emergency use, not yet licensed by the FDA for general commercial marketing. The other right guarantees individuals the right to refuse participation in potentially deadly drugs without incurring a penalty or losing a benefit to which they are otherwise entitled.

13. In early 2020, the nation, and the world, were faced with a novel coronavirus called SARS-CoV-2, which caused the highly contagious disease named COVID-19.

14. On January 31, 2020, the Secretary of Health and Human Services (HHS) issued a declaration of a public health emergency. The President declared a national emergency on March 13, 2020, all of which led to the development of investigational new drugs designed to perform as a vaccine from the virus, i.e., cause the body to produce antibodies to the virus so that the person is immune from infection when exposed to the true virus.

15. In order to implement the nationwide distribution and administration of these investigational new drugs, the U.S. Food and Drug Administration (FDA) issued an Emergency Use Authorization pursuant to 21 U.S.C. 360bbb-3 (Section 564 of the Food, Drug & Cosmetic Act.)

16. The FDA made clear on its website:

> FDA believes that terms and conditions of an EUA issued under Section 564 preempt state or local law, both legislative requirement and common-law duties, that impose different or additional requirements on the medical product for which the EUA was issued in the context of the emergency declared under section 564…In an emergency, it is critical that the conditions that are part of the EUA or an order or waiver issued pursuant to section 564A – those that FDA has determined to be necessary or appropriate to protect public health – be strictly followed, and no additional conditions be imposed.

17. In August 2020, the Centers for Disease Control (CDC) published the transcript of a meeting of the Advisory Committee on Immunizations and Respiratory Diseases, at which Dr. Amanda Cohn stated (@1:14:40):

> I just wanted to add that, just wanted to remind everybody, that **under an Emergency Use Authorization, an EUA, vaccines are not allowed to be mandatory**. So, early in the vaccination phase, individuals will have to be **consented** and **they won't be able to be mandated.** (Emphasis added)

18. In 2021, individuals nationwide, exercising a federally secured right to refuse investigational medical products, were subjected to unconstitutional treatment by authorities disagreeing with the individual's chosen option. Those individuals were not allowed to enjoy the equal protection of laws. They were subjected to severe human rights abuses violating ratified treaties, federal laws, and the laws of all US States and Territories.

19. In August 2021, Governor Kate Brown, the Oregon Health Authority, and its Director, Patrick Allen, usurped the authority of the United States Congress by issuing an administrative rule in defiance of federal law when they mandated the use of investigational new drugs by healthcare workers. Moreover, these Defendants engaged in outrageous tyrannical conduct by mandating that healthcare facilities deny employment to healthcare workers who exercised their federally secured legal right to refuse investigational drugs.

20. PeaceHealth, CEO Liz Dunne, as PeaceHealth's policymaker, Todd Salnas, Chief Executive of PeaceHealth Oregon Network, and CPE Doug Koekkoek, decided that the suffering of the few was justified by the windfall such suffering had on PeaceHealth's financial bottom line. Thus, PeaceHealth prescribed its own "required conditions" in defiance of Senator Edward Kennedy, Congress, and the rights of individuals under their authority as secured by the Constitution.

21. In August 2021, CEO Dunne and Todd Salnas, Chief Executive of PeaceHealth Oregon Networks, issued a despicable illegal mandate that shocked the conscience. During the height of the pandemic, when hospitalization rates soared, and SARS-CoV-2 variants abounded, CEO Dunne subjected 16,000 employees to investigational drug use under threat of penalty and outside of their free will and voluntary consent. Should employees not comply with CEO Dunne's fraudulent usurpation of authority, they were to be segregated, penalized, humiliated, and

eventually terminated from employment, thus violating the federally secured rights of those individuals to refuse an investigational drug without penalty and causing harm to the ability of the hospital to provide a quality standard of healthcare to communities within the state of Oregon.

22. Oregon Health Authority Director, Patrick Allen, used his office as official cover in hopes of obtaining for himself immunity from liability in future legal actions. Similarly, hiding behind the PREP Act as a liability cover, PeaceHealth, its policymaker, CEO Dunne, Todd Salnas, and CPE Koekkoek willfully chose to engage in violations of federal law. Defendants' wanton conduct mirrors the abuses of power perpetrated against humanity that led Senator Kennedy and Congress to act in the early 1970s. Yet, half a century later, humanity in the United States still suffers due to the willingness of persons and entities such as Defendants to violate the lawful Constitutional ideal of treating all persons equally before the law.

## II. Jurisdiction and Venue

23. This Court has original jurisdiction under 28 U.S.C. §§ 1331 and 1343.

24. The civil-rights portions of this action raise federal questions under the Spending Clause and the Fourteenth Amendment to the U.S. Constitution.

25. This Court has original jurisdiction under 42 U.S.C. §§ 1983 and 1988.

26. This Court has the authority to award costs and reasonable attorney's fees under 42 U.S.C. § 1988.

27. This court has supplemental jurisdiction over Plaintiff's state law claims.

28. This Court has personal jurisdiction over Defendants as they are domiciled within this Court's jurisdictional boundaries.

29 This Court has subject matter jurisdiction over the parties because all acts complained of herein were committed by Defendants in the State of Oregon and caused damage and/or deprivation to the Plaintiffs listed herein.

30. Venue is proper in this court because all events underlying the claims in this Complaint occurred in the State of Oregon, which is situated within this Court's jurisdiction, and all Defendants reside in the State of Oregon.

## III. Plaintiffs

31. The following individuals are plaintiffs herein:

31.1. Plaintiff, Hannah Bernhardt, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.2. Plaintiff, Sara Boysen, is an adult individual who all times pertinent resided in the State of Michigan, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.3. Plaintiff, Alisha Carter, is an adult individual who all times pertinent resided in the State of Iowa, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.4. Plaintiff, Katherine Derrick, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.5. Plaintiff, Sarah Guimont, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.6. Plaintiff, Kathy Murphy, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.7. Plaintiff, Dale Kingston, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.8. Plaintiff, Christina O'Harra, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.9. Plaintiff, Christina Shaneyfelt, is an adult individual who all times pertinent resided in the State of Florida, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.10. Plaintiff, Deborah Shaneyfelt, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.11. Plaintiff, Ann Doreen Waters, is an adult individual who all times pertinent resided in the State of Oregon, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

31.12. Plaintiff, Micaela West, is an adult individual who all times pertinent resided in the State of Tennessee, and was previously an employee of PeaceHealth or one of its DBA entities in Oregon.

## IV. Defendants

32. **Defendant PEACEHEALTH is a not-for-profit healthcare system headquartered in Clark County, WA** recognized as tax-exempt pursuant to Section 501(c)(3) of the Internal Revenue Code.

33. Defendant, Liz Dunne, is the President and Chief Executive Officer of PeaceHealth. Ms. Dunne is named as a defendant in her official and individual capacities.

34. Defendant, Doug Koekkoek, is the Chief Physician and Clinical Executive of PeaceHealth. Mr. Koekkoek is named as a defendant in his official and individual capacities.

35. Defendant, Todd Salnas, is the Chief Executive of PeaceHealth Oregon Network. Mr. Salnas is named as a defendant in his official and individual capacities.

35.1 Defendant, Patrick Allen, is the Director of the Oregon Health Authority. Mr. Allen is named as a defendant in his official and individual capacities.

35.2. Defendant, Kate Brown, is the Governor of the State of Oregon. Ms. Brown is named as a defendant in her official and individual capacities.

## V. History and Facts

36. Plaintiffs make no assertions regarding:

A. whether it is lawful for a private employer to mandate taking a ***licensed*** vaccine,

B. the safety or efficacy of any drug, biologic, or medical device, or

C. natural immunity versus drug-induced immunization.

37. Plaintiffs adamantly assert, however, that an individual has the federally secured right to refuse the administration of an Emergency Use Authorization (EUA) drug (e.g., Pfizer BioNTech COVID-19 Vaccine), biologic, or device (e.g., EUA testing articles and masks) without incurring a penalty or losing a benefit to which they are otherwise entitled.

38. Additionally, Plaintiffs assert that they have the federally secured right to refuse any activity or product under the PREP Act authority.

39. Because the EUA statute was created to allow the Secretary of HHS to authorize the use of a product for a purpose for which it is not already licensed, medical countermeasure products fall under the investigational or experimental classification by statute.[1]

40. Because EUA products are, by definition, used only during times of emergency, the laws regulating these products are not litigated as much as more commonly used statutes, so a brief recitation of the origin and history of these laws should prove helpful.

41. The aforementioned 1974 National Research Act established the National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research[2] (hereinafter referred to as the "Commission").

42. Congress required the Commission to:

A. "conduct a comprehensive investigation and study to identify the basic ethical principles which should underlie the conduct of biomedical and behavioral research involving human subjects,"

B. "develop guidelines which should be followed in such research to assure that it is conducted in accordance with such principles," and

C. "make recommendations to the [HHS] Secretary" for "such administrative action as may be appropriate to apply such guidelines to biomedical and behavioral research conducted or supported under programs administered by the Secretary."

[1] 21 U.S.C. §360bbb-3(a)(2)(A) and (B); See also the May 10, 2021, Scope of Authorization letter issued to Pfizer wherein the FDA advises Pfizer that its product is "an investigational vaccine not licensed for any indication." The same is true for the Moderna and Jannsen injections.

[2] Title II of the National Research Act - https://www.govinfo.gov/content/pkg/STATUTE-88/pdf/STATUTE-88-Pg342.pdf

43. Congress further required the Commission to consider "the nature and definition of informed consent in various research settings."[3]

44. On April 18, 1979, the Commission published its findings in the Federal Register in a report titled, "The Belmont Report."[4]

## VI. The Belmont Report

45. The Belmont Report outlined what the Commission considered "the nature and definition of informed consent" as follows:

A. "An autonomous person is an individual capable of deliberation about personal goals and acting under the direction of such deliberation. To respect autonomy is to give weight to autonomous persons 'considered opinions and choices while refraining from obstructing their actions…" (Emphasis added);

B. "To show lack of Respect for an autonomous agent is to repudiate that person's considered judgments, to deny an individual the freedom to act on those considered judgments…"(Emphasis added);

C. "Respect for persons requires subjects, to the degree that they are capable, be given the opportunity to choose what shall or shall not happen to them. This opportunity is provided when adequate standards for informed consent are satisfied" (Emphasis added).

46. The Belmont Report defined those adequate standards of informed consent as follows:

A. An agreement to participate in research constitutes valid consent only if voluntarily given. This element of informed consent requires conditions free of coercion and undue influence; (Emphasis added)

B. Coercion occurs when an overt threat of harm is intentionally presented by one person to another in order to obtain compliance;

[3] National Research Act Title II - PROTECTION OF HUMAN SUBJECTS OF BIOMEDICAL AND BEHAVORIAL RESEARCH Part A Section 202. (a)(1)(B)(iv)

[4] The National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research. - Belmont Report. Washington, DC: U.S. Department of Health and Human Services,1979

C. Undue influence, by contrast, occurs through an offer of an excessive, unwarranted, inappropriate, or improper reward or other overture in order to obtain compliance. Also, inducements that would ordinarily be acceptable may become undue influences if the subject is especially vulnerable;

D. Unjustifiable pressures usually occur when persons in positions of authority or commanding influence -- especially where possible sanctions are involved -- urge a course of action for a subject," (emphasis added), and;

E. …undue influence would include actions such as manipulating a person's choice through the controlling influence of a close relative and threatening to withdraw health services to which an individual would otherwise be entitled.

47. The Commission determined that if an individual is under outside pressure to participate in an investigational medical activity, then obtaining that individual's informed consent was legally impossible.

48. Congress mandated in the National Research Act that "[i]f the Secretary determines that administrative action recommended by the Commission should be undertaken by him, he shall undertake such action as expeditiously as is feasible."

49. Congress required the HHS Secretary to act upon the Commission's recommendations as outlined in the Belmont Report by establishing regulations to protect humans involved in biomedical research activities. **Therefore, given the complexity, the intent of Congress was not to draft those laws but to allow the HHS Secretary to promulgate regulations on its behalf** to protect humans involved with investigational drugs.

50. In the early 1980s, HHS acted upon the Commission's recommendations stating, "Based on the Belmont Report and other work of the National Commission, HHS revised and

expanded its regulations for protecting human subjects…The HHS regulations are codified at 45 Code of Federal Regulations (CFR) 46, subparts A through D."[5]

## VII. 45 CFR 46

51. 45 CFR Part 46 is entitled, "Protection of Human Subjects." Subpart A is entitled, "Basic HHS Policy for Protection of Human Research Subjects" and establishes that (a) the policy (for protection of human research subjects) "applies to all research involving human subjects conducted, supported, or otherwise subject to regulation by any Federal department or agency..." (Emphasis added).[6]

52. HHS designed a very broad definition of research when, at 45 CFR § 46.102 (Definitions): "Research means a systematic investigation, including research development, testing, and evaluation, designed to develop or contribute to generalizable knowledge. Activities that meet this definition constitute research for purposes of this policy, whether or not they are conducted or supported under a program that is considered research for other purposes"[7] (emphasis added). Research under this policy includes medical chart reviews by students or periodic studies of medical products under 21 U.S.C. §360bb-3 authorization.[8]

53. A "human subject" is broadly defined as (1) a living individual, (2) from whom data is obtained and used,[9] and (3) from whom identifiable private information is known.[10]

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[5] 45 CFR 46 FAQs. HHS.gov. Published 2018. Accessed May 18, 2023. https://www.hhs.gov/ohrp/regulations-and-policy/guidance/faq/45-cfr-46/index.html
[6] 45 CFR 46.101(a)
[7] 45 CFR 46.102(l)
[8] https://www.hhs.gov/ohrp/sites/default/files/human-subject-regulations-decision-charts-2018-requirements.pdf
[9] 45 CFR 46.102(e)(1)(i)
[10] 45 CFR 46.102(e)(1)(ii)

54. HHS regulations define[11] the term "human subject" at 45 CFR 46.102(e) as follows:

> (1) ***Human subject*** means a living individual about whom an investigator (whether professional or student) conducting research:
>
> > (i) Obtains information or biospecimens through intervention or interaction with the individual, and, uses, studies, or analyzes the information or biospecimens; or
> >
> > (ii) Obtains, uses, studies, analyzes, or generates identifiable private information or identifiable biospecimens.
>
> (2) ***Intervention*** includes physical procedures by which information or biospecimens are gathered (*e.g.,* venipuncture) and manipulations of the subject or the subject's environment that are performed for research purposes.
>
> (3) ***Interaction*** includes communication or interpersonal contact between investigator and subject.
>
> (4) ***Private information*** includes information about behavior that occurs in a context in which an individual can reasonably expect that no observation or recording is taking place, and information that has been provided for specific purposes by an individual and that the individual can reasonably expect will not be made public (e.g., a medical record).
>
> (5) ***Identifiable private information*** is private information for which the identity of the subject is or may readily be ascertained by the investigator or is associated with the information.
>
> (6) ***An identifiable biospecimen*** is a biospecimen for which the identity of the subject is or may readily be ascertained by the investigator or is associated with the biospecimen. (Emphasis in original.)

55. Congress drafted broad definitions for "research" and "subjects" to comply with the recommendations of the Belmont Report, which declared that "the general rule is that if there is any element of research in an activity, that activity should undergo review (third-party review to

[11] "Coded Private Information or Biospecimens Used in Research (2018)." HHS.gov. Published January 19, 2018. https://www.hhs.gov/ohrp/coded-private-information-or-biospecimens-used-research.html#:~:text=Identifiable%20private%20information%20is%20private,is%20associated%20with%20the%20information (Last accessed June 5, 2023)

ensure the health and rights of involved individuals are protected) for the protection of human subjects"[12] (emphasis added).

56. Therefore, if individuals are administered an investigational medical product and their private identifiable information is collected along with the details of their interaction with the product, and that information is monitored, studied, or analyzed for purposes of adding to the generalizable knowledge of the product, then the activity meets the definition of "research," thus requiring 45 CFR 46 compliance when the federal government is involved.

57. HHS ensured that all research activities would comply with Belmont Report's ethical requirements: (1) "Department or agency heads retain final judgment as to whether a particular activity is covered by this policy, and this judgment shall be exercised consistent with the ethical principles of the Belmont Report"[13] (emphasis added), (2) if the activity is considered exempt from the policy, then "the alternative procedures to be followed are consistent with the principles of the Belmont Report."[14]

58. Congress expressly prohibits the federal government from administering an investigational product to an individual without also complying with the ethical principles of the Belmont Report.

59. Placing an individual under a "sanction" for refusing an EUA drug, biologic, or device patently violates the ethical principles of the Belmont Report.

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[12] The Belmont Report Part A: Boundaries Between Practice & Research. "Research and practice may be carried on together when research is designed to evaluate the safety and efficacy of a therapy. This need not cause any confusion regarding whether or not the activity requires review; the general rule is that if there is any element of research in an activity, that activity should undergo review for the protection of human subjects."

[13] 45 CFR § 46.101(c)

[14] 45 CFR § 46.101(i)

60. The intent of Congress was to give the Belmont Report the force of law through 45 CFR 46 and the Federal Wide Assurance agreement (see discussion, *infra*) for the explicit purpose of protecting humans when they are offered a federally funded EUA investigational product.

61. To further protect Americans from medical research abuses in the future, Congress declared that, "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied." (45 CFR § 46.122)

62. Moreover, Congress also prohibited the United States Military from abusing individuals again by enacting 10 U.S.C. § 980(a), which provides in pertinent part, "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless — (1) the informed consent of the subject is obtained in advance."

63. 45 CFR 46 and the Belmont Report are conditions established by Congress for the express purpose of benefiting third-party beneficiaries (i.e., individuals considering the use of investigational medical products). The primary condition intended to benefit the third party is the requirement of authorities to prospectively obtain the third party's "legally effective informed consent" before administering to them federally funded investigational drugs (e.g., Pfizer-BioNTech COVID-19 Vaccine).

64. Therefore, pursuant to 45 CFR 46, "research" occurs when an individual is administered an investigational drug, the individual's private identifiable information is known, and data collected regarding their interaction with the drug is added to the generalizable knowledge about the drug.

65. When the federal government involves itself in research, it is required by law to comply with 45 CFR 46 and the ethical principles of the Belmont Report.

66. When the federal government determines that the activity does not require 45 CFR 46 oversight, then the activity is still required by federal law to comply with the ethical principles of the Belmont Report.

67. Moreover, federal research activity that is not deemed research for any other federal agency, statute, or regulation must still abide by 45 CFR 46 if the activity meets the definition of research set forth in 45 CFR 46.

68. The COVID-19 CDC Vaccination Program is a research activity requiring 45 CFR 46 compliance. (See *infra*)

69. At no time may the federal government administer an investigational medical product to an individual if their "legally effective informed consent" is not obtained in advance.

## VIII. Legally Effective Informed Consent

70. 45 CFR § 46.116 sets forth the Belmont Report's "adequate standards" of informed consent[15], and they include, but are not limited to:

> (a)(1) Before involving a human subject in research covered by this policy, an investigator shall obtain the legally effective informed consent of the subject or the subject's legally authorized representative; (Emphasis added)
>
> (a)(2) An investigator shall seek informed consent only under circumstances that provide the prospective subject or the legally authorized representative sufficient opportunity to discuss and consider whether or not to participate and that minimize the possibility of coercion or undue influence; (Emphasis added)

[15] The Belmont Report and 45 CFR §46.116 contain the only definition for what Congress deems legally effective informed consent. Therefore, when statutes explicitly or implicitly mandate a person to give their legally effective informed consent, these definitions must be understood as the intent of Congress for compliance purposes.

(a)(3) The information that is given to the subject or the legally authorized representative shall be in language understandable to the subject;

(a)(4) The prospective subject or the legally authorized representative must be provided with the information that a reasonable person would want to have in order to make an informed decision about whether to participate, and an opportunity to discuss that information;

(a)(5) Informed consent must begin with a concise and focused presentation of the key information that is most likely to assist a prospective subject or legally authorized representative in understanding the reasons why one might or might not want to participate in the research;

(a)(6) No informed consent may include any exculpatory language through which the subject or the legally authorized representative is made to waive or appear to waive any of the subject's legal rights;

(a)(7) For research involving more than minimal risk, an explanation as to whether any compensation and an explanation as to whether any medical treatments are available if injury occurs…;"

(a)(8) A statement that participation is voluntary, refusal to participate will involve no penalty or loss of benefits to which the subject is otherwise entitled, and the subject may discontinue participation at any time without penalty or loss of benefits to which the subject is otherwise entitled" (Emphasis added).

71. Legally Effective Informed Consent, according to the Belmont Report, can be broken down into its basic formula as (1) the individual must not be under outside pressure to participate, (2) the only reason an individual participates is that he or she believes the product may benefit their personal health goals, and (3) the conditions of 1 and 2 were established before the individual participated in the investigational product.

72. Only when authorities comply with 45 CFR 46 and the ethical principles of the Belmont Report can an opportunity exist for an individual to give their legally effective informed consent according to 45 CFR § 46.116(a)(1).

73. Informed Consent must be legally effective and prospective, according to HHS.

74. 45 CFR 46 applies to all federal agencies, departments, and the military (45 CFR § 46.101(a)). Additionally, twenty federal agencies incorporated 45 CFR 46 specifically into their regulatory framework.[16]

75. Through the Federal Wide Assurance (FWA) agreement (see *infra*), all U.S. States and Territories (i.e., state health agencies have FWA agreements) have agreed to comply with 45 CFR 46 and the Belmont Report's ethical guidelines.

76. Consensual medical experimentation involving investigational medical products can only exist under conditions that ensure individuals are free from outside pressures to participate.

77. Therefore, individuals have the explicit right to refuse an investigational drug, biologic, or device without incurring a penalty or losing a benefit to which they are otherwise entitled. When Defendants penalized Plaintiffs for refusing the administration of drugs undergoing clinical trials, they failed to comply with their duties to obtain the legally effective informed consent of Plaintiffs. Moreover, Defendants violated federal law and Plaintiff's federally secured rights via 45 CFR 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, and the CDC COVID-19 Vaccination Provider Agreement (see discussion, *infra*) by failing to obtain Plaintiffs' legally effective informed consent.

78. The failure to obtain Plaintiffs' legally effective informed consent is evidenced in Defendants' EUA drug mandates containing pre-published sanctions for persons refusing participation. (See discussion, *infra*)

[16] https://www.hhs.gov/ohrp/regulations-and-policy/regulations/common-rule/index.html

## IX. COVID-19 Research Activities

79. PeaceHealth and the State of Oregon are in a symbiotic relationship to conduct 45 CFR 46 research activities pertaining to COVID-19 EUA drugs, biologics, and devices on behalf of the federal government. Moreover, they are in a symbiotic relationship to obtain legally effective informed consent from individuals offered participation in those investigational medical products.

80. The federal government's Executive Branch purchased all COVID-19 EUA drugs and, in conjunction with HHS and the CDC, developed research activities that States and CDC Vaccination Program Providers must conduct on its behalf.[17]

81. Drugs, biologics, and devices authorized under 21 U.S.C. §360bbb-3 (see discussion, *infra*) are classified by the FDA as investigational (experimental)[18],[19] according to their labeling. They have no legal indication to treat, cure, or prevent any disease according to their labeling.

82. Moreover, if a product is already licensed by the FDA for its intended emergency use, the FDA is prohibited from issuing an EUA. (21 U.S.C. §360bbb-3(c)(3))

83. The only COVID-19 drugs made available to Plaintiffs are classified by the FDA as investigational new drugs. No FDA-licensed COVID-19 vaccines have been introduced into

[17] Persons must volunteer to participate in 21 U.S.C. §360bbb-3 or PREP Act activities. Therefore, governors, hospitals, manufacturers, distributors, etc. must voluntarily agree to abide by the terms and conditions of the product's Scope of Authorization and associated laws. Those terms require research activities pursuant to 45 CFR 46.

[18] Investigational new drug means, "A substance that has been tested in the laboratory and has been approved by the U.S. Food and Drug Administration (FDA) for testing in people…Also called experimental drug, IND, investigational agent, and investigational drug." NCI Dictionary of Cancer Terms. National Cancer Institute. Published 2023. Accessed June 25, 2023. https://www.cancer.gov/publications/dictionaries/cancer-terms/def/investigational-new-drug

[19] 21 CFR 312.3 (Definitions and Interpretations): See "Investigational new drug" and "Clinical investigation" Note that "clinical investigation" is distinct from "clinical trial." While all clinical trials are clinical investigations, not all clinical investigations are clinical trials.

commerce for general commercial marketing since the declaration of the pandemic in March 2020, through the filing of this Complaint.

84. A "marketed drug" is not the same as an "investigational drug."

85. A "marketed drug" is one that is licensed by the FDA for general commercial marketing and approved with an indication and usage for the treatment of a particular disease, which, via federal statute, EUA medical countermeasure products must not be. (See 21 USC 355a, *et seq*, 21 USC 360bbb-3(a)(2)(a,b))

86. Investigational new drugs are legally regulated entirely differently than licensed drugs. The FDA declared in its August 23, 2021 EUA to Pfizer that "Pfizer-BioNTech COVID-19 Vaccine" drug is legally distinct from its licensed "COMIRNATY" drug.

87. The distinction lies within the drug's classifications as assigned to them by the FDA. Those distinctions have significant legal consequences for the end user. (See discussion, *infra*)

88. EUA drugs, by their statutory definitions, are not licensed by the FDA for general commercial marketing and have no legal indication to treat, cure, or prevent any known disease.

89. Investigational drug "means a new drug or biological drug that is used in a clinical investigation." (21 CFR 312.3 "Investigational new drug")

90. Clinical investigation "means any experiment in which a drug is administered or dispensed to, or used involving, one or more human subjects. For the purposes of this part, an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice." (21 CFR 312.3 "Clinical investigation") (Emphasis added).

91. Only the FDA is authorized by Congress to assign a drug, biologic, or device its classification for purposes of regulation.

92. Drugs are governed by their classification and not by their formulation.

93. Congress explicitly drafted laws governing investigational new drugs to prevent the executive branch from continuing its history of abusing the rights of individuals who are administered investigational medical products.

94. On December 11, 2020, the FDA issued to Pfizer-BioNTech the first COVID-19 EUA for its investigational drug (officially named Pfizer-BioNTech COVID-19 Vaccine[20]), and the FDA confirmed that Pfizer's product "is an investigational vaccine not licensed for any indication."[21]

95. On December 18, 2020, the FDA issued to ModernaTX, Inc., an EUA for its investigational drug (officially named Moderna COVID-19 Vaccine), and the FDA confirmed that Moderna's product "is an investigational vaccine not licensed for any indication."[22]

96. On February 27, 2021, the FDA issued to Janssen Biotech, Inc., an EUA for its investigational drug (officially named Janssen COVID-19 Vaccine), and the FDA confirmed that Janssen's product "is an investigational vaccine not licensed for any indication."[23]

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[20] *Id*. The FDA improperly allowed Pfizer to add the word "Vaccine" to its investigational name. The court should not confuse this name to mean the drug's legal indication. Pfizer-BioNTech COVID-19 Vaccine is an investigational drug having no legal indication to treat, cure, or prevent any known disease. The FDA classified the drug as an "investigational new drug."

[21] 86 Fed.Reg. 5200, Jan. 19, 2021

[22] 86 Fed.Reg. 5211, Jan. 19, 2021

[23] 86 Fed.Reg. 28608, May 27, 2021

97. 21 U.S.C. §360bbb-3 requires the Secretary of HHS to "[a]ppropriate conditions for the monitoring and reporting of adverse events associated with the emergency use of the product (research activity)."

98. The Secretary establishes the conditions under which the research activities will occur in each EUA letter, known as the Scope of Authorization.

99. As an example, on January 19, 2021[24] the Secretary established mandatory conditions that Pfizer and emergency stakeholders (distributors, manufacturers, etc.) must follow, which involve 45 CFR 46 research activities.

100. Under the EUA's "Conditions of Authorization," the Secretary mandates in part:

* * *

> F. Pfizer Inc. will report to Vaccine Adverse Event Reporting System (VAERS):
>
> - Serious adverse events
> - Cases of Multisystem Inflammatory Syndrome in children and adults
> - Cases of COVID-19 that result in hospitalization or death, that are reported to Pfizer, Inc.
>
> G. Pfizer Inc. must submit to Investigational New Drug application (IND) number 19736 periodic safety reports at monthly intervals, within 15 days after the last day of a month…Each periodic safety report is required to contain descriptive information, which includes:
>
> - A narrative summary and analysis of adverse events submitted during the reporting interval, including interval

[24] Authorizations of Emergency Use of Two Biological Products During the COVID-19 Pandemic; Availability. Federal Register. Published January 19, 2021. Accessed June 7, 2023. https://www.federalregister.gov/documents/2021/01/19/2021-01022/authorizations-of-emergency-use-of-two-biological-products-during-the-covid-19-pandemic-availability

and cumulative counts by age groups, special populations (e.g., pregnant women), and adverse events of special interest.
- Newly identified safety concerns in the interval.

* * *

N. Pfizer Inc. will conduct post-authorization observational study(ies) to evaluate the association between Pfizer-BioNTech COVID-19 Vaccine and a pre-specified list of adverse events of special interest, along with deaths and hospitalizations, and severe COVID-19. The study population should include individuals administered the authorized Pfizer-BioNTech COVID-19 Vaccine under this EUA in the general U.S. population (16 years of age and older), populations of interest such as healthcare workers, pregnant women, immunocompromised individuals, subpopulations with specific comorbidities.

* * *

T. Vaccination providers administering Pfizer-BioNTech COVID-19 Vaccine must report the following information…to VAERS…:

- Serious adverse events
- Cases of Multisystem Inflammatory Syndrome in children and adults
- Cases of COVID-19 that result in hospitalization or death

101. VAERS reported 1,562,008 entries from December 2020 through May 26, 2023, including 35,272 deaths (1.6 per hour) and 263,462 (12.11 per hour) serious injuries. These numbers demonstrate historical entries for a drug and the vast involvement of the medical community to add to the "generalizable knowledge" of the product.

102. Healthcare providers and Pfizer, Moderna, and Janssen must identify the person receiving the product, monitor their involvement with the product, and report whether or not they had an adverse reaction to the product.

103. As of August 2021, when Defendants issued their mandates, all COVID-19 drugs were undergoing clinical trials and were already under an Institutional Review Board, which must comply with 45 CFR 46 and the FWA (see discussion *infra*).

104. COVID-19 drug manufacturers and government agencies use collected data to add to the generalizable knowledge about the product. These conditions meet 45 CFR 46, FWA, and the Belmont Report definitions of research activities.

105. The CDC Provider Agreement (see discussion, *infra*), EUA authorizations, and CDC's Advisory Committee on Immunization Practices (ACIP) recommendations demonstrate how the nationwide COVID-19 vaccination program is to be systematically investigated.

106. The federal government purchased all COVID-19 drugs and created the CDC COVID-19 Vaccination Provider Agreement for the administration of its property to individuals desiring to participate in the product. The Provider Agreement establishes additional research activities that Defendants must conduct on the government's behalf. The Provider Agreement states that Defendants "must administer COVID-19 Vaccine in accordance with all requirements and recommendations of CDC and CDC's Advisory Committee on Immunization Practices (ACIP)."

107. ACIP's Morbidity and Mortality Weekly Report from September 2021 confirms that in addition to "initial clinical trial data, ACIP…considered…real-world vaccine effectiveness studies, and post-authorization vaccine safety monitoring," information came from entities that executed the CDC Vaccine Provider Agreement and submitted the below-described information because the ONLY way to have authority to administer the COVID-19 Vaccines is by executing

the CDC Vaccine Provider Agreement.[25] The use of this information by ACIP demonstrates how the data collected "contributes to generalizable knowledge."

108. The ACIP recommendations[26] referenced in Footnote 1 of the CDC Provider Agreement[27] instruct Defendants to:

A. Provide an EUA Fact Sheet to potential recipients before being administered the drug.

B. Counsel potential vaccine recipients about expected systemic and local reactogenicity.

C. Follow additional clinical considerations, including details of administration and use in special populations (e.g., persons who are pregnant or immunocompromised or who have severe allergies) based on advice from the CDC (https://www.cdc.gov/vaccines/covid-19/info-by-manufacturer/ pfizer/clinical-considerations.html)

D. Adverse events that occur in a recipient after receipt of COVID-19 vaccine should be reported to the Vaccine Adverse Events Reporting System (VAERS).

E. Report vaccination administration errors, serious adverse events, cases of multisystem inflammatory syndrome, and cases of COVID-19 that result in hospitalization or death after administration of COVID-19 vaccine under EUA.

F. Report any clinically significant adverse event, whether or not it is clear that a vaccine caused the adverse event.

G. Inform vaccine recipients about V-Safe, the CDC's vaccine safety monitoring system that the CDC says "helps us monitor the safety of COVID-19 vaccines for everyone."[28]

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[25] ACIP, Morbidity and Mortality Weekly Report, "Use of Pfizer-BioNTech COVID-19 Vaccine in Persons Aged ≥ 16 Years: Recommendations of the Advisory Committee on Immunization Practices – United States, September 2021", Vol.70, No.38, p. 1344.

[26] *Id*., at 1347.

[27] The CDC Provider Agreement, at p.2, makes the ACIP Recommendations mandatory by the following language: "This agreement expressly incorporates all recommendations, requirements, and other guidance that this agreement specifically identifies through footnoted weblinks. Organization must monitor such identified guidance for updates. Organization must comply with such updates."

[28] https://www.cdc.gov/coronavirus/2019-ncov/vaccines/safety/pdfs/v-safe-information-sheet-508c.pdf

109. The CDC Provider Agreement further instructs Defendants:

A. Within 24 hours of administering a dose of COVID-19 Vaccine, record in the vaccine recipient's record and report required information to the relevant state, local or territorial public health authority.

B. Submit Vaccine-Administration Data through either (1) the immunization information system (IIS) of the state or local territorial jurisdiction or (2) another system designated by CDC according to CDC documentation and data requirements.

C. Organization must preserve the record for at least 3 years following vaccination, or longer if required by state, local, or territorial law. Such records must be available to any federal, state, local, or territorial public health department to the extent authorized by law.

D. Report the number of doses of COVID-19 Vaccine that were unused, spoiled, expired, or wasted as required by the relevant jurisdiction.

E. Provide a completed COVID-19 vaccination record card to every COVID-19 vaccine recipient.

110. Based on the detailed, organized, and methodical way HHS and the CDC structured the nationwide COVID-19 Vaccination Program, it meets the criteria for "a systematic investigation…designed to develop or contribute to generalizable knowledge."

111. The federal government's purchase of all COVID-19 EUA drugs (see discussion, *infra*), demonstrates that it is automatically bound to 45 CFR 46 via 45 CFR § 46.122, 10 U.S.C. § 980, and the Belmont Report.

112. When Congress established a **required condition** of the Secretary under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) "to ensure that individuals to whom the product is administered are informed "of the option to accept or refuse administration of the product," it intended compliance with 45 CFR §46.116 and the Belmont Report.

113. The nationwide CDC COVID-19 Vaccination Program authorized under 21 U.S.C. §360bbb-3 has elements of research activity requiring adherence to 45 CFR 46 and the Belmont Report. The statutes and regulations governing the research activities provide individuals who are considering whether or not to participate the statutory authority to accept or refuse without incurring a penalty or losing a benefit to which they are otherwise entitled.

114. Moreover, the COVID-19 immunization program is bound by statute to obtain the legally effective informed consent of the individual prospectively.

## X. ICCPR Treaty

115. In 1992, the United States Senate ratified the International Covenant on Civil and Political Rights Treaty (ICCPR).[29] Article VII states, "No one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment. In particular, no one shall be subjected without his free consent to medical or scientific experimentation." (Emphasis added)

116. Subjected means to be under the rule of law by one's authority.

117. Free consent means to be free from outside pressures to participate.

118. The U.S. Senate issued a resolution stating, "That the United States considers itself bound by Article 7 to the extent that 'cruel, inhuman or degrading treatment or punishment 'means the cruel and unusual treatment or punishment prohibited by the Fifth, Eighth and/or Fourteenth Amendments to the Constitution of the United States."[30]

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[29] Treaty Document 95-20 - INTERNATIONAL COVENANT ON CIVIL AND POLITICAL RIGHTS. (2023, May 19). https://www.congress.gov/treaty-document/95th-congress/20/all-info

[30] See "Resolution" - Treaty Document 95-20 - INTERNATIONAL COVENANT ON CIVIL AND POLITICAL RIGHTS. Congress.gov. Published 2023. Accessed June 5, 2023. https://www.congress.gov/treaty-document/95th-congress/20/all-info

119. The U.S. Senate considered it to be a violation of Article 7 of the ICCPR Treaty and the 5th Amendment's Due Process Clause if individuals were forced to forfeit liberty and property without due process for refusing medical experimentation. The Senate also considered it to be a violation Article 7 of the ICCPR Treaty and the 14th Amendment's Equal Protection Clause when individuals who refused medical experimentation were treated differently than those who accepted medical experimentation.

120. The United States Senate stated that Articles One through Twenty-Seven of the ICCPR Treaty are not "self-executing" but "that it is the view of the United States that States Party to the Covenant should, wherever possible, refrain from imposing any restrictions or limitations on the exercise of the rights recognized and protected by the Covenant, even when such restrictions and limitations are permissible under the terms of the Covenant."

121. Treatment by authorities debasing an individual's liberty, autonomy, and human dignity, for the express purpose of coercing that individual to surrender their Constitutional rights leading to feelings of fear, anguish, and inferiority, meets the international definition of cruel, inhumane, and degrading treatment or punishment.[31]

122. Whereas the "United Nations Convention Against Torture and Other Cruel, Inhuman, or Degrading Treatment or Punishment" treaty deals specifically with physical torture or the threat of physical torture, Article VII of the ICCPR Treaty speaks to the political actions of

[31] "Treatment that humiliates or debases an individual, showing a lack of respect for, or diminishing, their human dignity, or when it arouses feelings of fear, anguish or inferiority capable of breaking an individual's moral and physical resistance." - degrading treatment or punishment. Published 2023. Accessed June 6, 2023. https://home-affairs.ec.europa.eu/networks/european-migration-network-emn/emn-asylum-and-migration-glossary/glossary/degrading-treatment-or-punishment_en

governments and the laws of governments leading to a loss of rights, safety, and liberty, or the feelings that such actions will lead to those losses.

123. The UN Human Rights Committee spoke to Article IV of the ICCPR Treaty regarding the derogation of rights when states declare an emergency. "Article 4, paragraph 2, of the Covenant explicitly prescribes that no derogation from the following articles may be made: article 6 (right to life), article 7 (prohibition of torture or cruel, inhuman or degrading punishment, or of medical or scientific experimentation without consent.)"[32] (Emphasis added.)

124. Article 4.2 of the ICCPR Treaty established the restriction of derogation of informed consent rights as a peremptory norm, potentially opening Defendants up to criminal liability for issuing and executing mandates involving non-consensual medical experimentation.[33]

125. It cannot be reasonably disputed that Defendants subjected individuals under their authority to medical experimentation outside of their free will and voluntary consent. When Plaintiffs refused to participate, Defendants intentionally inflicted "cruel," "inhumane," and "degrading" treatment in violation of federal law and the International Covenant on Civil and Political Rights Treaty.

126. Specific examples of the cruel, inhumane, and degrading treatment that PeaceHealth inflicted upon the Plaintiffs herein include employment termination, leave without pay, mandatory EUA COVID testing (even for teleworkers), public humiliation, defamation of

[32] "No justification or extenuating circumstances may be invoked to excuse a violation of article 7 for any reasons, including those based on an order from a superior officer or public authority." - Human Rights Committee in its General Comment No. 20 on article 7 (A/44/40)

[33] General comment no. 29 states of emergency (article 4) GE.01-44470 (E) 190901
GENERAL COMMENT ON ARTICLE 4 (adopted at the 1950th meeting, on 24 July 2001)

character, segregation, isolation, gaslighting, emotional trauma, financial devastation, and forced public disclosure of private medical and employment information.

127. PeaceHealth's COVID-19 immunization mandate, relying exclusively on investigational medical products, violated each plaintiff's Constitutional rights under the Fourteenth Amendment, federal statutes under 21 U.S.C. § 360bbb-3, and 45 CFR 46, Article VII of the ICCPR Treaty, the Belmont Report, and their benefits under the CDC COVID-19 Vaccination Program Provider Agreement.

128. No treaty, Constitutional authority, federal statute, regulation, or state law exempts Defendants from their duty of ensuring that Plaintiffs are not under political, legal, financial, social, or other degrading pressures to take a COVID-19 investigational drug.

## XI. 21 U.S.C. §360bbb-3 (aka Section 564)

129. Congress expressly prohibits any person from introducing into commerce a drug, biologic, or medical device not licensed by the FDA for general commercial marketing (21 U.S.C. §355(a)) to ensure individuals are protected against medical research abuses.

130. Investigational drugs, biologics, and devices are strictly controlled by Congress. Only authorized persons may access, distribute, and administer the investigational products and only under the prescribed conditions established by Congress.

131. However, over time, Congress recognized the need to allow individuals to access unlicensed products for various medical reasons (also known as "expanded access protocols"). Therefore, Congress established 21 U.S. Code §360bbb, titled "Expanded Access to Unapproved Therapies and Diagnostics."

132. Numerous conditions must be met before the legal administration of products authorized pursuant to the section can occur. The overriding requirement, irrespective of the granted expanded access protocol, is that the individual must give their legally effective informed consent, whether the consent is under written or verbal conditions.

133. Making it patently clear of their intent to protect Americans from medical research abuses, Congress enacted legislation prohibiting federal funding for research activities if the informed consent obtained from the individual is not legally effective nor prospective for the civilian (45 CFR § 46.122) and for the military (10 U.S.C. §980).

134. Section 564 of the Food, Drug, and Cosmetic Act ("FDCA"), 21 U.S.C §360bbb-3[34] authorizes the HHS Secretary to grant emergency expanded access protocols to (1) FDA-licensed products for unlicensed uses or (2) products the FDA has not licensed for general commercial marketing.

135. Congress requires the HHS Secretary to establish "appropriate conditions designed to ensure that individuals to whom the product is administered are informed" of:

> (ii)(II) the significant known and potential benefits and risks of such use, and of the extent to which such benefits and risks are unknown;
>
> (ii)(III) the option to accept or refuse administration of the product;
>
> (ii)(III) the consequences, if any, of refusing administration of the product, and
>
> (ii)(III) the alternatives to the product that are available and of their benefits and risks.[35]

[34] 21 U.S.C. §360bbb-3 is commonly referred to as section 564.
[35] 21 U.S.C. 360bbb-3(e)(1)(A)

136. Informing the individual of the product risks, alternatives, benefits, and health consequences provides that individual with the quality information required to give legally effective informed consent.[36]

137. Congress requires healthcare professionals to inform the individual of "the option to accept or refuse administration of the product," meaning the healthcare professional is required by Congress to inform the individual of his or her legal rights under 21 U.S.C. §360bbb-3.

138. A legal right is a power held by an individual resulting from a constitution, statute, regulation, or judicial precedent of which no other authority may interfere unless prescribed in law.

139. There are two legal rights conferred upon individuals considering whether to participate in a Section 564 medical countermeasure product, which are (1) the right to **accept** a Section 564 medical product, and (2) the right to **refuse** to take or use a Section 564 medical product.

140. The decision belongs exclusively to the individual and must be under conditions free of outside pressures. If individuals are under outside pressure to participate, then it is legally impossible for them to give their free consent; thus, their rights have been infringed upon.

141. Therefore, the right to accept or refuse an EUA medical countermeasure is absolute, and no authority may infringe upon that right. This understanding becomes vitally important when

[36] The requirements of informing the subject of risks, benefits, alternatives, and health consequences, and that the Secretary has authorized the use of the investigational drug mirrors 45 CFR §46.116 requirements.

viewed in the light of Congress preempting state laws for PREP Act products and the COVID-19 Provider Agreement PeaceHealth signed with the Centers for Disease Control (CDC)(see *infra*.)

142. There are three specific persons upon whom Congress confers a right under Section 564, which are:

A. the HHS Secretary, who is empowered to authorize access to investigational drugs, biologics, or medical devices and the conditions under which that access can occur,

B. the healthcare professional who is authorized to inform the individual of their Section 564 legal rights and to administer unlicensed Section 564 medical products, and

C. the individual who is authorized to accept or refuse Section 564 medical products.

143. Congress established a required condition that "[w]ith respect to the emergency use of an unapproved product, the Secretary, to the extent practicable given the applicable circumstances described in subsection (b)(1), shall, for a person who carries out any activity for which the authorization is issued, establish such conditions on an authorization under this section as the Secretary finds necessary or appropriate to protect the public health." (21 USC 360bbb-3(e))

144. Additionally, Congress conferred authority onto the Secretary so that he may "appropriate conditions on who may administer the product with respect to the emergency use of the product, and on the categories of individuals to whom, and the circumstances under which, the product may be administered with respect to such use."

145. These "appropriate conditions" and the "circumstances" are outlined in the Emergency Use Authorization (EUA) letter issued to the manufacturer of the emergency medical countermeasure under the "Scope of Authorization."

146. Therefore, the Scope of Authorization contained in each EUA letter has the force of law as it establishes the conditions under which the emergency activities can occur, prescribing duties for the manufacturer and rights of all persons involved in the administrative process of the product.

147. The Secretary determined that the COVID-19 pandemic required healthcare workers to provide individuals contemplating the use of one of the EUA products with a drug fact insert sheet *before the product is administered* to act as a function of informed consent. In other words, the Secretary thought it was practical that every person be afforded this right and, as such, mandated that requirement under the Scope of Authorization for each EUA.

148. To ensure individuals are protected when they are offered EUA medical products, Congress was explicit in that "[n]othing in this section [21 U.S.C. 360bbb-3] provides the Secretary any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section (21 U.S.C. 360bbb-3(l))." (Emphasis added)

149. Congress, therefore, prohibits governments (e.g., governors, mayors, school boards) and voluntary participants (e.g., hospitals, manufacturers, doctors) from having the authority to require any person to participate in any 21 U.S.C. §360bbb-3 activity, at any time, under any statute, regulation, or state policy or custom.

150. The explicit purpose of this statutory restriction is to ensure that no person is under a "sanction," "coercion," "undue influence," or "unjustifiable pressure"[37] to participate. If individuals are under those pressures, then no federal funds could be expended for the

[37] The Belmont Report's conditions that would nullify legally effective informed consent.

administration of an EUA product, nor could any healthcare provider acting on behalf of the federal government obtain an individual's Legally Effective Informed Consent.

151. The individual has the right to accept the product, and the healthcare professional has the authority to administer the product, but neither is required to act on the demands of the other. Congress established a guideline requiring both the healthcare professional and the individual to mutually agree to the process to meet the legal requirement of 21 U.S.C. §360bbb-3.

152. The purpose of this requirement is to ensure that the individual receives a quality standard of healthcare even under emergency conditions because not everyone is a proper candidate to take or use an investigational medical countermeasure.

153. Therefore, if the HHS Secretary is the only person authorized to establish the conditions under which persons can take or use EUA medical products, **then by what authority did Defendants amend the Scope of Authorization and require that which Congress prohibits**?

154. Therefore, when Defendants established a policy requiring Plaintiffs to inject a COVID-19 EUA investigational drug into their bodies as a condition to enjoy employment and State benefits, they were required by federal law to ensure that (1) licensed products existed to meet the legal requirements of the mandate, and (2) Plaintiffs were to be informed that they were under no obligation to take unlicensed COVID-19 EUA drugs, nor would they incur a penalty or lose a benefit when refusing to participate in the PREP Act activity. Defendants did neither.

155. Defendants decided to violate federal law, contractual duties, and the Fourteenth Amendment rights of Plaintiffs when they issued a mandate having a compliance date before a licensed COVID-19 vaccine was available.

## XII. HHS EUA Precedent

156. On January 28, 2005, HHS issued the first EUA[38] under its new Section 564 authority. The military requested EUA protocols for Anthrax Vaccine Adsorbed (AVA), to be utilized by civilians and service members. HHS stated, "The issuance of this Authorization for the emergency use of AVA is the first time that the EUA authority is being used. FDA intends to explain clearly the reasons for each issuance, termination, or revocation of an EUA. The agency wishes to make its decision-making understandable to help ensure that members of the public, and particularly those individuals who may be eligible to receive a medical product authorized for emergency use, are informed about the basis of an EUA determination."

157. HHS mandated that individuals participating in the AVA investigational product must be informed of the following statements:

A. Individuals (service members and civilians) who refuse anthrax vaccination will not be punished. (Emphasis added)

B. Refusal may not be grounds for any disciplinary action under the Uniform Code of Military Justice.

C. Refusal may not be grounds for any adverse personnel action. Nor would either military or civilian personnel be considered non-deployable or processed for separation based on refusal of anthrax vaccination.

D. There may be no penalty or loss of entitlement for refusing anthrax vaccination,

E. This information shall read in the trifold brochure provided to potential vaccine recipients as follows: You may refuse anthrax vaccination under the EUA, and you will not be punished. No disciplinary action or adverse personnel action will be taken. You will not be processed for separation, and you will still be deployable.

[38] https://www.govinfo.gov/content/pkg/FR-2005-02-02/pdf/05-2028.pdf

> There will be no penalty or loss of entitlement for refusing anthrax vaccination.[39]

158. The explicit instructions in the EUA language directly relate to AVA's classification as an investigational new drug not licensed by the FDA for any legal indication. Moreover, the language was designed to ensure that healthcare professionals could obtain the legally effective informed consent of the individual because it expressly informed the individual that no "sanction" would be imputed for refusal, thus nullifying all outside pressures to participate. No amendments to Section 564 have altered its requirements since HHS issued this first EUA.

159. The reason HHS was crystal clear about an individual's right to refuse an investigational drug was to respect court orders.

## XIII. Judicial EUA Precedent

160. On October 27, 2004, U.S. District Court Judge Sullivan spoke to the individual's authority to refuse investigational drugs without consequence when he held in *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004), that:

> A. Congress has prohibited the administration of investigational drugs to service members without their consent. This Court will not permit the government to circumvent this requirement; and,
>
> B. Unless and until FDA properly classifies AVA [an anthrax vaccine] as a safe and effective drug for its intended use, an injunction shall remain in effect prohibiting defendants 'use of AVA on the basis that the vaccine is either a drug unapproved for its intended use or an investigational new drug within the meaning of 10 U.S.C. §1107. Accordingly, the involuntary anthrax vaccination program, as applied to all persons, is rendered illegal absent informed consent or a Presidential waiver." (Emphasis added.)

[39] Federal Register/Vol. 70, No. 21/Wednesday, February 2, 2005/Notices 5455 IV Conditions of Authorization

161. Immediately upon Judge Sullivan's ruling, the Department of Defense ended all punitive activities against service members and civilian employees because the federal court affirmed the individual's statutory authority to refuse investigational drugs without consequence. Except for 10 U.S.C. § 1107, the laws leading Judge Sullivan to his ruling apply to individuals irrespective of civilian or military service. No laws have changed to negate Judge Sullivan's 2004 ruling.

162. Judge Sullivan added clarity to the importance of what was argued before the court by stating: "The Court is persuaded that the right to bodily integrity and the importance of complying with legal requirements, even in the face of requirements that may potentially be inconvenient or burdensome, are among the highest public policy concerns one could articulate." *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004).

163. *Doe* and the HHS provide judicial and administrative precedent affirming the right of individuals to refuse investigational products without incurring a penalty or losing a benefit to which they are otherwise entitled. Nothing in the law has changed to nullify that right since those precedents were firmly established.

## XIV. Federal Wide Assurance (FWA)

164. In 2001, HHS created the Office of Human Rights Protection (OHRP), which established the Federal Wide Assurance (FWA) agreement. The FWA is an agreement by entities conducting business with HHS to comply with 45 CFR 46 and the Belmont Report's ethical guidelines.

165. HHS states, "The Federal Wide Assurance (FWA) is an assurance of compliance with the U.S. federal regulations for the protection of human subjects in research. It is approved

by the Office for Human Research Protections (OHRP) for all human subjects research conducted or supported by the U.S. Department of Health and Human Services (HHS). The FWA is also approved by OHRP for federal wide use, which means that other U.S. federal departments and agencies that have adopted the U.S. Federal Policy for the Protection of Human Subjects (also known as the Common Rule) may rely upon the FWA for the research they conduct or support. An FWA is the only type of assurance currently accepted and approved by OHRP. It is required whenever an Institution becomes engaged in human subjects research conducted or supported by any U.S. federal department or agency that has adopted the Common Rule…"[40]

166. The OHRP assigns an FWA identification number to entities (hereinafter referred to as "Contracting Provider") that fulfill application requirements. An FWA identification number is issued only after the legally binding agreement between the Contracting Provider and the United States government has been signed.

167. The FWA's main purpose is to benefit a third-party beneficiary because the FWA agreement authorizes the Contracting Provider to participate in federally funded programs involving humans with investigational drugs if, and only if, the Contracting Provider agrees to protect the health and legal rights of the third-party beneficiaries (i.e., humans who are administered investigational drugs, biologics, or devices under the research conditions described above).

168. The fact that the entire FWA agreement hinges upon the intended rights of third-party beneficiaries means that Contracting Providers have a duty to the third-party beneficiaries under the terms of the FWA agreement.

[40] Office for Human Research Protections. Federal Wide Assurance Instructions. HHS.gov. Published January 7, 2011. Last accessed May 19, 2023.

169. The intended benefit to the third-party beneficiary is the right to accept or refuse participation in investigational products, clinical trials, and other research activities without fearing consequences for refusal and to know that independent Institutional Review Boards will provide oversight, ensuring their health, safety, and rights are protected.

170. Although the third-party beneficiaries are not signatories to the agreement, they are the intended third-party beneficiaries of the agreement, and their rights were violated the moment PeaceHealth and Oregon penalized Plaintiffs for refusing to take EUA products (i.e., investigational drugs, and testing articles).

171. The FWA agreement requires the Contracting Provider to ensure that no third-party beneficiary is under outside pressure to participate in an investigational drug, biologic, or medical device.

172. The FWA agreement requires PeaceHealth to assure potential participants that they will not incur a penalty or lose a benefit to which they are otherwise entitled when refusing participation.[41]

173. The benefits to which potential participants are otherwise entitled include, but are not limited to:

A. continued employment,

B. 14th Amendment rights,

C. 5th Amendment rights,

D. paid time off,

E. bonuses,

[41] "The Federal Wide Assurance (FWA) is the only type of assurance currently accepted and approved by OHRP. Through the FWA, an institution commits to HHS that it will comply with the requirements in the HHS Protection of Human Subjects regulations at 45 CFR part 46." - HHS. 45 CFR 46.116(b)(8) requires the individual to be informed they will not be penalized for refusing participation in a research activity.

F. raises,

G. health insurance,

H. 401k contributions,

174. The duty placed upon the Contracting Provider is owed to those who refuse as well as those who accept the administration of investigational drugs.

175. Therefore, when Defendants punished third-party beneficiaries (Plaintiffs) for refusing the administration of an investigational drug, Defendants:

A. activated the terms and conditions of the contract,

B. violated the terms of the contract causing injury to the rights of the third-party beneficiary,

C. created a cause of action for breach of contract in favor of the third-party beneficiary.

176. The Fourteenth Amendment's Equal Protection Clause provides additional protections by requiring all persons involved in federally funded COVID-19 countermeasure programs to be treated equally before the law.

177. Defendants violated the Equal Protection Clause by only punishing persons choosing one of two federally secured options (refuse).

178. Defendants violated the Equal Protection Clause when they coordinated with the Oregon State Employment Security Department to deny unemployment benefits to Plaintiffs solely on the basis of workers exercising a federally secured option of refusing administration of an investigational drug, thus violating the Unconstitutional Conditions Doctrine.

179. The Unconstitutional Conditions Doctrine reflects the U.S. Supreme Court's repeated pronouncements that the government "may not deny a benefit to a person on a basis that infringes his constitutionally protected interests."[42]

180. The U.S. Supreme Court has held: "For at least a quarter-century, this Court has made clear that even though a person has no 'right 'to a valuable governmental benefit and even though the government may deny him the benefit for any number of reasons, there are some reasons upon which the government may not rely. It may not deny a benefit to a person on a basis that infringes his constitutionally protected interests—especially, his interest in freedom of speech. For if the government could deny a benefit to a person because of his constitutionally protected speech or associations, his exercise of those freedoms would, in effect, be penalized and inhibited. This would allow the government to 'produce a result which (it) could not command directly.'"[43] "Such interference with constitutional rights is impermissible."[44]

181. Based on longstanding Supreme Court precedent, the government is prohibited from denying individuals the right to receive unemployment benefits because the individual exercised his or her constitutionally protected right to bodily autonomy by refusing investigational drug administration. Such actions plainly violate the Fourteenth Amendment rights of equal protection of Plaintiffs.

182. Intentionally blank.

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[42] *Koontz v. St. Johns River Water Mgmt. Dist.*, 570 U.S. 595, 133 S. Ct. 2586, 186 L. Ed. 2d 697, 24 Fla. L. Weekly Fed. S 435 (2013)
[43] *Perry v. Sindermann*, 408 U.S. 593 (1972), citing *Speiser v. Randall*, 357 U.S. 513, 526, 78 S.Ct. 1332, 1342, 2 L.Ed.2d 1460.
[44] *Id.*

183. PeaceHealth enacted an illegal company policy violating federal laws resulting in Liz Dunne making false claims to[45] the United States Government and the State of Oregon. PeaceHealth then informed the State that Plaintiffs violated the company policy when exercising a federally secured right to refuse investigational new drugs under 21 U.S.C. §360bbb-3 protocols. When determining Plaintiffs' rights to unemployment benefits, the State ignored federal law and the U.S. Constitution. Instead, the State acted on a custom and chose to impose sanctions and withhold benefits to "transform private predilections into compulsory rules of behavior no less than legislative pronouncements,"[46] thus violating the Unconstitutional Conditions Doctrine.

184. The State denying employment benefits to Plaintiffs on the sole basis of refusing 21 U.S.C. §360bbb-3 countermeasure products demonstrates State policy and custom (see *infra*) under which PeaceHealth acted when retaliating against Plaintiffs exercising their 21 U.S.C. §360bbb-3 option to refuse.

## XV. PREP Act & Section 564 Preemption of State Law

185. In 2005, Congress passed the Public Readiness and Emergency Preparedness Act, hereafter referred to as the PREP Act (42 USC 247d-6d and 42 USC 247d-6e), to provide immunities for persons volunteering for "covered" activities. Accordingly, the HHS Secretary has issued a COVID-19 PREP Act declaration at 85 FR 15198.

186. The first provision of the PREP Act (42 USC 247d-6d) is entitled "Targeted liability protections for pandemic and epidemic products and security countermeasures."

[45] The CDC COVID-19 Vaccination Provider Agreement signed by PeaceHealth plainly states that, "Non-compliance with the terms of Agreement may result in suspension or termination from the CDC COVID-19 Vaccination Program and criminal and civil penalties under federal law, including but not limited to the False Claims Act, 31 U.S.C. § 3729 *et seq*., and other related federal laws, 18 U.S.C. §§ 1001, 1035, 1347, 1349."
[46] *Adickes v. S. H. Kress & Co*., 398 U.S. 144 (1970)

187. The second provision of the PREP Act (42 USC 247d-6e) is entitled "Covered countermeasure process."

188. Congress expressly crafted language preempting state law (42 USC 247d-6d(b)(8)), which provides, in pertinent part:

> (8) Preemption of State law
>
> > During the effective period of a declaration under subsection (b)…no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that—
> >
> > (A) is different from, or is in conflict with, any requirement applicable under this section; and
> >
> > (B) relates to the…administration…of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.].

189. Congress also expressly established that the plan to administer a covered countermeasure (i.e., any EUA COVID-19 investigational drug) shall be voluntary. Specifically, Congress stated the following at 42 USC 247d-6e(c), in pertinent part:

> (c) Voluntary program
>
> The Secretary shall ensure that a…Department of Health and Human Services plan to administer or use a covered countermeasure is consistent with any declaration under 247d–6d of this title…and that potential participants are educated with respect to contraindications, the voluntary nature of the program, and the availability of potential benefits and compensation under this part. [Emphasis added.]

190. The purpose of 21 U.S.C. §360bbb-3 informing the individual of "the significant known…risks of such use, and of the extent to which such benefits and risks" and of "the alternatives to the product that is available and of their benefits and risks" is because the individual is not only consenting to be injected with an investigational drug, but they must also consent to participate in a legally binding agreement under the terms and conditions established by Congress.

191. Individuals who consent to receive one of the COVID EUA investigational drugs must agree to the following terms and conditions, including but not limited to:

A. forfeiture of civil litigation rights resulting from injuries;[47]

B. allowing their private identifiable information to be collected and used for a variety of purposes by unknown persons;[48]

C. allow their involvement with the EUA product to be cataloged by various persons for unknown purposes,

D. allow the data collected about their adverse events to be utilized by researchers for unknown purposes and for eternity,[49]

E. agree to assume greater risks to their safety, health, and legal rights.[50]

192. Neither the State of Oregon nor PeaceHealth can enact laws, proclamations, or customs that coerce individuals to enter into a legally binding agreement outside of their free will and voluntary consent. In fact, Congress expressly preempted all state and local laws, regulations, and rules to ensure no individual would be coerced into participating in the federal government's legally binding agreement.

193. Congress was explicit when it pronounced that "no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that…is different from, or is in conflict with, any requirement applicable under this section; and…or to any matter included in a requirement applicable to…the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.]" (42 USC 247d-6d(b)(8),(A),(B)).

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[47] PREP Act forfeits all civil actions for damages in most situations.

[48] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to obtain private identifiable information.

[49] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to monitor, report and study a variety of adverse reactions to EUA products.

[50] Section 564 requires potential recipients to be made aware of the risks, alternatives, and the fact that the product is only authorized by the Secretary under emergency conditions. These elements provide potential recipients with the required information to make a quality and legally effective decision to consent. Therefore, consent means the individual agrees to assume more than minimal risk as defined above.

194. 21 U.S.C. §360bbb-3 is under 21 U.S.C. 301. Therefore, the PREP Act preemption extends to "the option to accept or refuse," which must only be under voluntary conditions.

195. Congress expressly preempted state laws interfering with the legal rights of individuals to decide whether or not to participate in the use of an EUA medical product under PREP Act authority. The preemption extends to at-will employment laws that private employers would otherwise utilize to interfere with an employee's option to accept or refuse without consequence.

196. A person cannot lawfully establish 21 U.S.C. §360bbb-3 conditions contrary to Congress and those laid out by the HHS Secretary's Scope of Authorization, and if they do so, the person would be usurping the authority of Congress to require that which Congress and the Secretary prohibit.

197. Congress, not the Defendants, determines who can participate in a PREP Act activity and the conditions under which they can participate. Defendants may not amend those conditions by usurping the authority of Congress.

198. Under § 247d-6d(b)(8)(A), "no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that…is different from, or is in conflict with, any requirement applicable under this section…and related to the…use…or administration…of the covered countermeasure…"

199. The FDA issued an opinion[51] regarding federal preemption of 21 U.S.C. §360bbb-3 (aka section 564):

[51]" Emergency Use Authorization of Medical Products and Related Authorities," Section VII. U.S. Food and Drug Administration. Published 2022. Accessed June 6, 2023. https://www.fda.gov/regulatory-information/search-fda-guidance-documents/emergency-use-authorization-medical-products-and-related-authorities#preemption

"FDA anticipates that conflicts between federal and state law may arise when FDA acts under sections 564, 564A, and 564B if states have existing requirements governing the shipment, holding, dispensing, administration, or labeling of unapproved medical products or approved medical products for unapproved uses. Courts have stated that the Supremacy Clause of the U.S. Constitution can operate to nullify both state legislative requirements and state common-law duties. Under the legal principles of implied conflict preemption, courts have found state law preempted where it is impossible to comply with both federal and state law, or when the state law 'stands as an obstacle to the accomplishment and execution of the full purposes and objectives of Congress.' Consistent with this case law, section 4(a) of Executive Order 13132 states that '[a]gencies shall construe... a Federal statute to preempt State law only where the statute contains an express preemption provision or there is some other clear evidence that the Congress intended preemption of State law, or where the exercise of State authority conflicts with the exercise of Federal authority under the Federal statute.' FDA states that the terms and conditions of an EUA issued under section 564 preempt state or local law, both legislative requirements and common-law duties, that impose different or additional requirements on the medical product for which the EUA was issued in the context of the emergency declared under section 564. Similarly, an order or waiver issued under section 564A and pre-positioning under section 564B preempt state or local law, both legislative requirements and common-law duties, that impose different or additional requirements related to the activity authorized under sections 564A or 564B. To the extent state or local law may impose requirements different from or

> in addition to those imposed by the EUA for a particular medical product within the scope of the declared emergency or threat of emergency (e.g., requirements on prescribing, dispensing, administering, or labeling of the medical product), such law 'stands as an obstacle to the accomplishment and execution of the full purposes and objectives of Congress, 'and 'conflicts with the exercise of Federal authority under [§ 564].' The same rationale applies to an order or waiver issued under section 564A and pre-positioning of an MCM under section 564B." (Emphasis added)

200. However, although no express preemption language exists within 21 U.S.C. §360bbb-3, the field preemption doctrine demonstrates that Congress created a pervasive regulatory scheme designed only to be regulated by the federal government. Moreover, state laws in conflict with the regulatory scheme of 21 U.S.C. §360bbb-3 pose an obstacle to the goals of the federal government's emergency countermeasure program.

201. State laws affording private employers legal authority to interfere with the federal statutory requirements of 21 U.S.C. §360bbb-3 and the PREP Act (e.g., at-will employment doctrine) violate the Supremacy Clause of the United States Government.

202. If Defendants can punish citizens for refusing to participate in a PREP Act activity or product, then they must also have the authority to punish citizens participating in a PREP Act activity or product. Under that scenario, one can easily see that the "option to accept or refuse" no longer belongs to the individual but to a third party. The resulting chaos and unconstitutional conditions in such a scenario are explicitly why Congress preempted the authority of any person to interfere with either federally secured option granted to individuals considering participation.

203. Defendants' mandate of involuntary participation in a PREP Act activity and their corresponding actions to coerce, threaten, and unduly pressure Plaintiffs to participate changes the "voluntary nature of the program" into an obligatory condition, depriving Plaintiffs of their federally protected authority "to accept or refuse" without consequence. The defendants' actions demonstrate that they attempted to unlawfully usurp the regulatory power of the United States government instead of properly implementing the emergency medical countermeasure protocols with which they voluntarily agreed to comply.

204. Congress expressly claimed preemption for the PREP Act and thereby 21 U.S.C. §360bbb-3. Thus Defendants are expressly prohibited from acting in a fiat manner to establish conditions regarding participation in the use of a covered countermeasure product contrary to federal statutes, agency regulations, and the Scope of Authorization outlined in the FDA-issued EUA letter to the manufacturer covering all EUA activities.

205. Those improper conditions include, but are not limited to:

A. coercing employees to surrender their federally secured right "of the option to accept or refuse administration of the [PREP Act] product,"

B. usurping the HHS Secretary's authority to "[a]ppropriate conditions on who may administer the product with respect to the emergency use of the product, and on the categories of individuals to whom, and the circumstances under which, the product may be administered with respect to such use," (Emphasis added)

C. requiring that which Congress explicitly prohibits: "Nothing in this section provides the Secretary any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section,"

D. interfering with the explicit requirement of Congress that "[t]he Secretary shall ensure that a State, local, or Department of Health and Human Services plan to administer or use a covered countermeasure (e.g., Pfizer BioNTech COVID-19 Vaccine)…are educated with respect to contraindications, the voluntary nature of the program…"(Emphasis added) (42 U.S. Code §247d-6e(c)).

206. Healthcare professionals must obtain legally effective informed consent from voluntary participants **specifically because the participants are denied relief for any damages resulting damages from the EUA product or activity**.

## XVI. CDC COVID-19 Vaccination Program Provider Agreement

207. The Centers for Disease Control (CDC) states that "[a]t this time, **all COVID-19 vaccine in the United States has been purchased by the U.S. government** (USG) for administration exclusively by providers enrolled in the CDC COVID-19 Vaccination Program and remains U.S. government property until administered to the recipient. Only healthcare professionals enrolled through a health practice or organization as vaccination providers in the CDC COVID-19 Vaccination Program (and authorized entities engaged in shipment for the Program) are authorized to lawfully possess, distribute, deliver, administer, receive shipments of, or use USG-purchased COVID-19 vaccine. Other possession, distribution, delivery, administration, shipment receipt, or use of COVID-19 vaccine outside the parameters of the Program constitutes, at a minimum, theft under 18 U.S.C. § 641, and violation of other federal civil and criminal laws. Violators are subject to prosecution to the full extent of the law." [See Exhibit A – CDC Covid-19 Vaccination Program Provider Agreement.]

208. Although the program states it is a "Vaccination Program" (hereinafter referred to as "CDC Vaccination Program"), the federal government has not distributed any FDA-licensed COVID-19 vaccines. Instead, it has relied exclusively on unlicensed COVID-19 EUA drugs for the program's administration.

209. Before the CDC accepts a person or entity as a Provider in the CDC Vaccination Program, that person or entity is required to sign the CDC COVID-19 Vaccination Program Provider Agreement (hereinafter referred to as the "Provider Agreement").

210. The Provider Agreement informs the person or entity that, "Your Organization's chief medical officer (or equivalent) and chief executive officer (or chief fiduciary)—collectively, Responsible Officers—must complete and sign the CDC COVID-19 Vaccination Program Provider Requirements and Legal Agreement (Section A)" (See Exhibit A.)

211. The Provider Agreement requires the organization to assign a person or persons who will be under a legal obligation to ensure the program is carried out effectively, declaring, "For the purposes of this agreement, in addition to Organization, Responsible Officers named below will also be accountable for compliance with the conditions specified in this agreement. The individuals listed below must provide their signature after reviewing the agreement requirements."

212. "This program is a part of collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC. To receive one or more of the publicly funded COVID-19 vaccines (COVID-19 Vaccine), constituent products, and ancillary supplies at no cost, Organization agrees that it will adhere to the following requirements…" (Emphasis added).

213. Therefore, the CDC clearly states that the Provider Agreement works in conjunction with "relevant state" and other municipality immunization agreements. This requirement denotes state action involving private parties acting in the capacity of a state actor.

214. HHS requires any entity conducting business with its organization to submit and be approved for a Federal Wide Assurance agreement (see discussion, *supra*) in advance of participating in any program involving humans with investigational drugs under its authority. The fact that the medical products in question are under an EUA does not exempt entities conducting business with HHS from first agreeing to obtain an FWA before participation. This fact is why the CDC chose only to distribute the program via State's existing immunization programs. Each state already has an HHS FWA agreement in place.

215. The Executive Branch of the government is required to comply with 21 U.S.C. § 360bbb-3 requirements and all other laws and regulations protecting humans involved in investigational medical products under emergency access protocols.

216. When the Executive Branch chose to purchase all COVID-19 vaccines (i.e., licensed and unlicensed COVID-19 drugs), it was required to ensure that all applicable laws associated with each drug's classification were adhered to by all volunteering participants.

217. The Executive Branch chose to establish the Provider Agreement as the mechanism to ensure those legal obligations were followed.

218. While the Provider Agreement does not replace the laws and regulations governing any EUA drug classification, it adds an extra layer of legal obligations required of volunteer participants.

219. PeaceHealth, Liz Dunne, Todd Salnas, and Doug Koekkoek (or their authorized equivalent) signed the Provider Agreement agreeing to comply with all "applicable laws" regarding EUA products and PREP Act activities.

220. PeaceHealth agreed to inform individuals under their authority, of their legal rights to accept or refuse EUA medical products.

221. PeaceHealth agreed to ensure individuals were not under outside pressures to participate in obtaining the legally effective informed consent of participants in the COVID-19 drugs.

222. PeaceHealth, by signing the Provider Agreement, agreed to waive the applicability of all state and local laws that are "in conflict" with the terms and conditions of the Provider Agreement or requirements applicable to the PREP Act and Section 564 protocols. The waiver

would include Governor Browns's Oregon Health Agency's unlawful COVID-19 mandate. (see *infra*)

223. PeaceHealth agreed to directly benefit individual potential vaccine recipients under their authority by signing the Provider Agreement.

224. The Provider Agreement requires that all volunteer participants:

A. "must provide an approved Emergency Use Authorization (EUA) fact sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative,"

B. "Organization must report moderate and severe adverse events following vaccination to the Vaccine Adverse Event Reporting System (VAERS),"

C. "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine,"

D. "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws."

225. The EUA Fact Sheet is required because the Executive Branch of the government is the sole sponsor of EUA products,[52] and federal law requires them to obtain the legally effective informed consent of each individual before the administration of the product. Moreover, the HHS Secretary requires each recipient to be given the Fact Sheet for each EUA COVID-19 investigational drug from which the federal branch of government cannot exempt itself. The required Fact Sheet acts as a function of the "informed consent" process for persons ascertaining whether or not they will participate in the EUA product.

226. The Executive Branch is required to report adverse events as part of the government's COVID-19 Vaccination Program because federal law requires this of every EUA

[52] The Federal government chose to purchase and retain ownership of all EUA COVID-19 drugs. However, that ownership does not negate their legal obligations under Section 564.

product, which the HHS Secretary echoed in each of the EUA letters issued to pharmaceutical companies. Moreover, the requirement to monitor, collect, and report, adverse reactions (research activities) from the drugs' use denotes how these products are governed by 45 CFR 46, requiring both IRB and Belmont Report compliance.

227. The requirement that the "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws" is because federal law declares:

A. "This policy does not affect any state or local laws or regulations (including tribal law passed by the official governing body of an American Indian or Alaska Native tribe) that may otherwise be applicable and that provide additional protections for human subjects" (45 CFR 46.101(f));

B. Additionally, federal law declares, "The informed consent requirements in this policy are not intended to preempt any applicable Federal, state, or local laws (including tribal laws passed by the official governing body of an American Indian or Alaska Native tribe) that require additional information to be disclosed in order for informed consent to be legally effective" (45 CFR 46.116(i));

C. This policy does not affect any foreign laws or regulations that may otherwise be applicable and that provide additional protections to human subjects of research (45 CFR 46.101(g)).

228. The Provider Agreement required PeaceHealth to acknowledge the law before acceptance, as follows: "By signing this form, I certify that all relevant officers, directors, employees, and agents of Organization involved in handling COVID-19 Vaccine understand and will comply with the agreement requirements listed above…" (Emphasis added)

229. Therefore, Defendants, at all times pertinent, agreed to participate in a joint effort to conduct research activities and obtain the legally effective informed consent of individuals on behalf of the United States Government when signing the CDC COVID-19 Vaccination Program Provider Agreement.

## XVII. Nature of Case

230. This is a case for damages against Governor Kate Brown, Oregon Health Authority Director, Patrick Allen, CEO Liz Dunne, Todd Salnas, Chief Executive Oregon Network, and CPE Doug Koekkoek, who at all times pertinent herein served as State Actors for illegally and unconstitutionally penalizing individuals exercising their statutory right to refuse an investigational drug without incurring a penalty or losing a benefit to which they are otherwise entitled. The State of Oregon had outsourced its Centers for Disease Control (CDC) emergency COVID-19 immunization program to PeaceHealth. Moreover, Governor Brown issued an executive order under color of law authorizing Oregon Health Authority to establish employment conditions based on COVID-19 vaccination status. As a result, Defendants acted under color of state law at all times pertinent and abridged Plaintiffs' constitutional rights of equal protection and due process when Plaintiffs refused to inject those investigational drugs into their bodies.

## XVIII. Statement Of Facts

231. PeaceHealth is a licensed "general hospital" provider under OAR 333-500-0040.

232. PeaceHealth's employees include healthcare professionals licensed by the State of Oregon.

233. The State of Oregon licenses, regulates, and oversees the operations of PeaceHealth.

234. PeaceHealth operates facilities throughout Oregon and is a participating immunization provider for the Oregon Health Authority's immunization program.

235. Immunization program administration constitutes a public function of the State of Oregon.

236. The Federal government owns all investigational and licensed COVID-19 drugs.

237. The executive branch of government established the CDC COVID-19 Vaccination Program to distribute its property (COVID-19 drugs) to volunteering participants.

238. The CDC works through existing state immunization programs whereby the State facilitates licensed medical facilities and healthcare professionals to administrate the federal government's COVID-19 program and administer the government's property under the PREP Act (42 USC 247d-6d) and 21 U.S.C. §360bbb-3 emergency authorization.

239. PeaceHealth willfully volunteered to participate in the CDC COVID-19 Vaccination Provider Program.

240. The CDC COVID-19 Vaccination Program constitutes an exclusive public function of the State.

241. Congress prohibits the HHS Secretary from requiring any person to participate in any activity that becomes lawful pursuant to 21 U.S.C. §360bbb-3 (i.e., the use of EUA products). Therefore, it also prohibits Defendants from requiring any person to participate in any activity, including any healthcare worker.[53]

242. The United States Constitution does not authorize Governor Kate Brown to amend treaties, federal statutes, and or federal agency regulations by fiat rule.

243. Governor Kate Brown was prohibited by 21 U.S.C. §360bbb-3 from amending the conditions established by the HHS Secretary regarding the administration of any emergency medical countermeasure program or product.

244. Governor Brown volunteered the State of Oregon to participate in the CDC COVID-19 Vaccination Program.

[53] The "activity that becomes lawful pursuant to 21 U.S.C. §360bbb-3" is the use of the EUA COVID-19 drugs.

245. Emergency Use Authorization (EUA) COVID-19 drugs have been classified by the FDA as investigational new drugs[54] and are not classified as "vaccines."[55]

246. Pursuant to its Federal Wide Assurance agreement with the federal government, Oregon State agreed to comply with 45 CFR Part 46 protocols and the ethical principles of the Belmont Report when it involved a human with an investigational drug. Thus, HHS assigned the State of Oregon Federal Wide Assurance Number FWA00000520 (under Oregon Health Authority), denoting a legally binding agreement between the State of Oregon and the United States Government for the explicit benefit of third-party participants.

247. Governor Brown was prohibited by Federal law from denying individuals the right to accept or refuse investigational new drugs funded by the Federal government or to apply consequences when accepting or refusing their administration.

248. The PREP Act and 21 U.S.C. §360bbb-3 preempt Oregon State laws, regulations, ordinances, and executive proclamations that interfere or conflict with the emergency medical countermeasure program.

249. PeaceHealth, via its FWA, IRB, 21 U.S.C. §360bbb-3, and the CDC Vaccination Program Provider Agreement, is prohibited by Federal law from denying individuals the right to accept or refuse investigational new drugs funded by the Federal government or applying consequences when accepting or refusing their administration.

250. PeaceHealth is prohibited via the PREP Act from mandating that any person participates in any activity that becomes lawful pursuant to Section 42 USC 247d-6d.

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[54] 21 CFR 312.3 "Investigational new drug" "means a new drug or biological drug that is used in a clinical investigation." 21 CFR 312.3 "Clinical investigation" "means any experiment in which a drug is administered or dispensed to, or used involving, one or more human subjects. For the purposes of this part, an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice."

[55] 26 U.S.C. § 4132(a)(2) "(2) Vaccine The term "vaccine" means any substance designed to be administered to a human being for the prevention of 1 or more diseases." EUA drugs do not have a legal indication for the "prevention" of any known disease.

252. PeaceHealth volunteered to act on behalf of the State to provide the public function of administering the State's COVID-19 emergency immunization program.

253. The State of Oregon and PeaceHealth are bound by law to conduct COVID-19 research activities, and also to obtain the legally effective informed consent of individuals, and ensure that the Fourteenth Amendment rights of Plaintiffs are protected and secured at all times pursuant to the administrative functions of the CDC COVID-19 Vaccination Program.

254. On August 13, 2021, Governor Kate Brown issued Executive Order 21-19, "COVID-19 Vaccination Requirements for State Executive Branch." The order was unlawful because it subjected citizens under her direct authority to use investigational drugs outside of their free will and voluntary consent. Governor Kate Brown unlawfully extended her authority by ignoring the Constitutional authority of Congress, and requiring "employees" of the State to inject an investigational drug into their bodies "on or before October 18, 2021" as a condition to continue employment and enjoy the rights afforded to them as a State licensed healthcare worker.

255. On August 13, 2021, the FDA had not licensed any COVID-19 investigational drug. Moreover, from August 13, 2021, through the date of filing of this Complaint, no COVID-19 drug licensed by the FDA has been introduced in the State of Oregon for general commercial marketing.

256. Governor Brown authorized State agencies to condition access to public benefits based solely on whether a person participated in investigational medical research activities. Governor Brown said, "Nothing in this order prohibits entities within the Executive Branch from implementing requirements that exceed the requirements of this Executive Order…"

257. Governor Brown held complete authority over the Oregon Health Authority (OHA) Board. Her office possesses the power to appoint the super majority of members and also to call

for their resignation at any time. She has the authority to take executive action to correct errant and illegal behavior stemming from OHA activities.

258. On August 05, 2021, OHA and its Director, Patrick Allen, acted on Governor Brown's authority and issued PH 34-2021 Temporary Administrative Order "Vaccination and Testing Requirements to Control COVID-19 for Healthcare Providers and Healthcare Staff." The rule violated federal law and Plaintiffs' Constitutional rights when mandating "health care personnel and healthcare staff who work in healthcare settings to either be vaccinated against COVID-19 or be tested for COVID-19 on at least a weekly basis." The violation is evident in that Defendants required persons to inject an unlicensed investigational drug into their bodies to comply with the mandate or to participate in an unlicensed investigational diagnostic device.

259. OHA engaged in misrepresentation of facts by presenting the investigational drugs which had no legal indication to treat, cure, or prevent any known disease as being classified by the FDA as a "vaccine." Drugs are only classified as a "vaccine" after receiving a marketing license and receiving a legal indication to prevent a specific disease from the FDA.

260. OHA's PH 34-2021 order was illegal because it subjected healthcare facilities and workers to investigational drug use outside of their free will and voluntary consent. Moreover, the testing articles OHA relied upon were also classified as investigational by the FDA and could not come under a mandate.

261. On August 25, 2021, OHA issued PH 38-2021, replacing PH 34-2021, removing the option to continue accessing living wages should the employee test using investigational diagnostic articles. The PH 38-2021 Temporary Administrative Rule states, "On or before October 18, 2021, healthcare providers and healthcare staff must provide their employer, contractor, or responsible party with either: Proof of vaccination showing they are fully vaccinated; or

Documentation of a medical or religious exception." This rule was a severe violation of ratified treaties, federal laws, State executive agreements, and Plaintiffs' constitutional and statutory rights. (See, *infra*)

262. On August 5, 2021 PeaceHealth, CEO Liz Dunne, Todd Salnas, and CPE Doug Koekkoek, conspiring with Governor Kate Brown and Patrick Allen, and acting under the color of law, unlawfully subjected Plaintiffs to investigational drug use under threat of penalty outside of Plaintiffs' free will and voluntary consent.

263. PeaceHealth mandated that Plaintiffs inject an investigational drug into their bodies, or they "may" face punitive actions (see, *infra*). The statement itself was a violation of PeaceHealth's CDC COVID-19 Vaccination Provider Agreement because it placed Plaintiffs under outside pressures to participate, nullifying their ability to give legally effective informed consent.

264. Defendants concealed at all times pertinent any reference to 21 U.S.C. § 360bbb-3 in their respective published mandates, demonstrating that Plaintiffs had the legal authority to refuse EUA drugs, biologics, or devices without consequence. This concealment was by design and not by chance.

265. Defendants intentionally usurped the federal government's authority by amending 21 U.S.C. § 360bbb-3 in an effort to cause Plaintiffs to surrender their Constitutional and statutory rights. Defendants later acted on their threats of penalty and punished Plaintiffs for refusing their unlawful mandates.

266. Defendants' actions led to:

A. a deprivation of both substantive and procedural due process rights under the Fourteenth Amendment,

B. a deprivation of equal protection rights under the Fourteenth Amendment,

C. a deprivation of rights guaranteed under the Spending Clause,

E. severe emotional distress,

F. outrageous conduct that shocks the conscience.

## XIX. State Action Doctrine

267. Because of the pervasiveness of control exerted by the State's COVID-19 emergency immunization program, Defendants acted under the color of law at all times pertinent.

268. The State of Oregon and PeaceHealth's COVID-19 immunization program constitutes an exclusive public function of the State because the CDC requires that its drugs be administered through a "collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC" as stated in the CDC Provider Agreement.

269. The State of Oregon and PeaceHealth's COVID-19 immunization program relies exclusively on investigational new drugs, the administration of which is governed by federal statutes enforceable under 42 U.S.C. § 1983.

270. In *Maine v. Thiboutot*, 448 U.S. 1 (1980), the court held that "Even were the language ambiguous, however, any doubt as to its meaning has been resolved by our several cases suggesting, explicitly or implicitly, that the § 1983 remedy broadly encompasses violations of federal statutory as well as constitutional law."

272. Additionally, in a decision dated June 8, 2023, the United States Supreme Court in *Health and Hospital Corporation of Marion Cty. V. Talevski*, 599 U.S. ___ (2023)[56], stated, "Although federal statutes have the potential to create §1983-enforceable rights, they do so under this Court's precedents only when the statute unambiguously confers those rights."

273. The *Talevski* court spoke to its method of determining a statute's § 1983 viability when it stated, "*Gonzaga* sets forth the Court's established method for ascertaining unambiguous

[56] Because the *Talevski* decision is so new, there was no page number assigned as of the date of the filing of this Complaint.

conferral. Courts must employ traditional tools of statutory construction to assess whether Congress has 'unambiguously conferred' 'individual rights upon a class of beneficiaries' to which the plaintiff belongs…Notably, it must be determined that 'Congress intended to create a federal right' for the identified class, not merely that the plaintiffs fall 'within the general zone of interest that the statute is intended to protect.' *Id*., at 283 (emphasis omitted). The test for unambiguous conferral is satisfied where the provision in question is 'phrased in terms of the persons benefited' and contains 'rights-creating,' individual-centric language with an 'unmistakable focus on the benefited class.' *Id*., at 284, 287 (emphasis omitted). If a statutory provision surmounts this significant hurdle, it 'secures individual rights that are deemed 'presumptively enforceable' under §1983."

274. 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) contains a required condition of the Secretary "to ensure that individuals to whom the product is administered are informed— of the option to accept or refuse administration of the product."

275. Therefore, the intended beneficiary class is those considering whether or not to be administered an EUA medical countermeasure. The "option to accept or refuse" is unambiguous conferral of power upon the individual considering participation.

276. 45 CFR 46 was developed by the express request of Congress to confer protective benefits for persons participating in medical research activities.

277. The CDC COVID-19 Vaccination Program Provider Agreement establishes required activities, including elements of medical research activity, thus requiring 45 CFR 46 compliance.

278. 45 CFR 46 §116 "uses clear rights-creating language, and speaks in terms of the persons benefited," and has an "unmistakable focus on the benefited class."

279. Article VII of the ICCPR Treaty states, in clear rights-conferring language, that no person may be subjected to medical experimentation without free consent. The FDA defines medical experimentation as "an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice." (21 CFR 312.3 "Clinical Investigation")

280. Defendant's FWA agreement and the use of IRBs also exist for the express benefit of the individual and contain rights-conferring language.

281. The Scope of Authorization for each Emergency Use Authorization requires healthcare professionals to provide potential recipients with a drug Fact Sheet insert stating, "Under the EUA, it is your choice to receive or not receive" the product. This statement is the Secretary affirming the individual's legal rights under 21 U.S.C. 360bbb-3.

282. The CDC COVID-19 Vaccination Program Provider Agreement requires "organizations" to "provide an approved Emergency Use Authorization (EUA) Fact Sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative." This requirement aims to inform the individual of his or her legally effective informed consent rights and to meet the required conditions under 21 U.S.C. §360bbb-3.

283. 45 CFR 46 §122 states, "Federal funds administered by a federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied." By direct nexus, this restriction qualifies as a rights-conferring language for any spending legislation enacted by Congress involving this section of law.

284. 10 U.S.C. § 980 states, "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless - the informed consent of the subject is obtained in advance."

285. Obtaining legally effective informed consent is a required legal condition, and it means that no individual can be under a "sanction," "coercion," "undue influence," or any other form of outside pressure to participate when considering whether or not to participate in a medical countermeasure.

286. The individual has the right, via the spending legislation (see, *infra*), among other statutes, to accept or refuse the available COVID-19 EUA medical products without fearing any repercussions whatsoever because the federal government fully funds the entire program. Therefore, the spending restriction statutes "use clear rights-creating language, and speak in terms of the persons benefited" and have an "unmistakable focus on the benefited class."

287. Although many rights-conferring items are contained in the above law provisions, the most notable is that no person can incur a penalty or lose a benefit when accepting or refusing an investigational medical product, as in any of the available COVID-19 EUA injections.

288. The purpose of those provisions directly relates to the Belmont Report's ethical principles of (1) "Respect for Person," (2) "Beneficence", and (3) "Justice," for persons involved in any research element. Every federal agency, department, and the military is required to comply with the Belmont Report. Moreover, no federal funds can be expended on research activities if the Belmont Report's ethical principles are not established in the research activity's protocol documents before the activity is even approved.[57]

289. The U.S. Supreme Court has held that "to act 'under color of' state law for § 1983 purposes does not require that the defendant be an officer of the State. It is enough that he is a willful participant in joint action with the State or its agents. Private persons, jointly engaged with

[57] PeaceHealth and Oregon Health Authority's FWA agreement are their promises to comply with the Belmont Report's ethical principles and adherence to 45 CFR 46.

state officials in the challenged action, are acting 'under color' of law for purposes of § 1983 actions."[58]

290. On August 13, 2021 Governor Brown issued unlawful official Proclamation 21-29 authorizing state agencies to require State employees to inject an investigational COVID-19 drug into their bodies as a condition to access public employment or other benefits.

291. On August 25, 2021 the Oregon Health Authority, under Director Patrick Allen's authority, conditioned access to healthcare employment within the State on Plaintiffs participating in the use of medical research drugs[59]:

A. "health care personnel and healthcare staff who work in healthcare settings to be vaccinated against COVID-19 or request a medical or religious exemption."

B. "After October 18, 2021"

1. "A health care provider or healthcare staff may not work, learn, study, assist, observe, or volunteer in a healthcare setting unless they are fully vaccinated…"

C. "Healthcare personnel includes individuals, paid and unpaid working, learning, studying, assisting, observing or volunteering in a healthcare setting providing direct patient or resident care.."

D. "Healthcare setting includes any place where health care, including physical or behavioral health care is delivered and includes, but is not limited to any health care facility or agency licensed under ORS chapter 441 or 443, such as hospitals…"

E. Fully Vaccinated "means having received both doses of a two-dose COVID-19 vaccine or one dose of a single-dose COVID-19 vaccine and at least 14 days have passed since the individual's final dose of COVID-19 vaccine."

292. At all times pertinent, Defendants did not provide Plaintiffs information on where they may receive an FDA-licensed COVID-19 vaccine. Rather, it is an unassailable fact that

[58] *Dennis v. Sparks*, 449 U.S. 24 (1980)
[59] See Exhibit D, Oregon Health Authority Temporary Administrative Order

Defendants relied exclusively on investigational COVID-19 drugs for compliance with their temporary administrative COVID-19 mandatory vaccination rules.

293. Therefore, Governor Kate Brown and Director Patrick Allen of the Oregon Health Authority utilized the powers of their offices to mandate that healthcare facilities terminate the employment of individuals working in healthcare settings should those individuals exercise their statutory authority to refuse the injection of a COVID-19 investigational drug. The mandate was unlawful the moment it was published.

294. PeaceHealth, CEO Liz Dunne, Todd Salnas, and CPE Doug Koekkoek, acting under the color of law, issued and acted on their mandate on August 30, 2021 stating in part:

A. "It is the policy of PeaceHealth to require healthcare workers (HCWs) to be fully vaccinated against COVID-19."

B. "HCWs are considered fully vaccinated two weeks after they have received Pfizer or Moderna second dose, Johnson & Johnson single dose or other U.S. Food and Drug Administration (FDA) Emergency Use Authorization (EUA), FDA approved, FDA authorized or World Health Organization (WHO) emergency use/approved COVID-19 vaccine/series."

C. "All new or returning HCWs must obtain a COVID-19 vaccination series/dose or submit a medical or religious exemption that is approved and can be accommodated before starting/restarting at PeaceHealth. If a new or returning HCW is scheduled to start before October 15, 2021, they may start/ restart at PeaceHealth if they are (1) fully vaccinated; (2) have an approved medical or religious exemption for which reasonable accommodations are made; or (3) if they have received at least one dose of a COVID-19 vaccination/series dose, have a second dose scheduled (if a two-dose series) and will be fully vaccinated by October 15, 2021."

295. However, that policy conflicts with a policy sent to Plaintiffs on August 03, 2021 via email[60] informing them that "PeaceHealth is implementing a new policy that requires all caregivers to show proof of having received a complete series of COVID-19 vaccine doses or

[60] See Attachment Email

submit a qualifying medical exemption, on or before August 31, 2021. Regardless, PeaceHealth acted under color of law at all times when issuing their unlawful policies that employees must participate in COVID-19 medical research activities using investigational COVID-19 drugs.

296. Therefore, by issuing Proclamation 21-29, Governor Kate Brown and, by direct nexus, Patrick Allen and the Oregon Health Authority, unlawfully extended their authority and mandated that healthcare facilities violate the constitutional and statutory rights of Plaintiffs. PeaceHealth engaged in that state action when issuing and acting on the State's COVID-19 vaccination policy.

297. Neither the State Proclamation nor PeaceHealth's COVID-19 policy provided for a legal mechanism to ensure the due process rights of Plaintiffs were protected before they were robbed of their liberty and property.

298. As stated by the U.S. Supreme Court in *Jackson v. Metropolitan Edison Co*., 419 U.S. 345 (1974), "We have, of course, found state action present in the exercise by a private entity of powers traditionally exclusively reserved to the State."

299. The State's immunization program is both an historical and exclusive public function of the state.

300. The COVID-19 emergency medical countermeasure program is an exclusively public function of the State.

301. The CDC COVID-19 Vaccination Provider Agreement is fully funded by the United States federal Government operating exclusively through the CDC. Therefore, every administered shot of any COVID-19 EUA drug is under 21 U.S.C. §360bbb-3, 45 CFR 46, 10 U.S.C. § 980, and the Belmont Report's requirements.

302. The U.S. Government initiated a contract with PeaceHealth and an agreement with Oregon to ensure those protocols were adhered to at all times pertinent.

303. The U.S. government must administer the CDC COVID-19 Vaccination Program in compliance with the 14th Amendment because it exclusively relies on the State's immunization programs to distribute and administer its property.[61] Therefore, the government's vaccination program has a Constitutional obligation to ensure individuals are not deprived of their 14th Amendment rights when considering participation in the State's joint COVID-19 vaccination program with the federal government. That protection is effectively secured when an individual is allowed to give their legally effective informed consent. Defendants refused Plaintiffs that right at all times pertinent.

304. The State and PeaceHealth freely volunteered to participate in the federal government's COVID-19 Vaccination Program.

305. PeaceHealth and the State are in a symbiotic relationship to obtain the legally effective informed consent of potential recipients for the success of the State's COVID-19 immunization program.[62]

[61] The Biden Administration purchased all available COVID-19 drugs and ordered that the drugs remain the property of the government until administered to a human. (See, *supra*)

[62] *Brunette v. Humane Society of Ventura County*, 294 F.3d 1205 (9th Cir. 2002): "*Burton* (*Burton v. Wilmington Pkg. Auth*, 365 U.S. 715, 81 S. Ct. 856 (1961)) teaches that substantial coordination and integration between the private entity and the government are the essence of a symbiotic relationship. Often significant financial integration indicates a symbiotic relationship. *See Rendell Baker,* 457 U.S. at 842-43, 102 S.Ct. 2764; *Vincent v. Trend W. Tech. Corp.,* 828 F.2d 563, 569 (9th Cir. 1987). For example, if a private entity, like the restaurant in *Burton,* confers significant financial benefits indispensable to the government's "financial success," then a symbiotic relationship may exist. *Vincent,* 828 F.2d at 569. A symbiotic relationship may also arise by virtue of the government's exercise of plenary control over the private party's actions. *See Dobyns v. E-Systems, Inc.,* 667 F.2d 1219, 1226-27 (5th Cir. 1982) (finding symbiotic relationship where the government-controlled a private peacekeeping force engaged in a government-directed field mission in the Sinai Peninsula).

306. The State did not provide PeaceHealth with an alternative process to comply with federal statutory obligations of obtaining legally effective informed consent of Plaintiffs when engaged in the State action on its behalf.

307. PeaceHealth is required to update the vaccination status of individuals on behalf of the State.

308. The State relies on State agents, such as PeaceHealth, to achieve the goals of the State's COVID-19 immunization program.

309. 100% of all private parties participating in the State's COVID-19 immunization program are "clothed with the authority of state law" to access investigational medical products, administer those medical products, and bill the government for those activities on behalf of State authority.[63]

310. Without the State, PeaceHealth could not participate in the CDC COVID-19 Vaccination Program Provider Agreement nor provide that governmental function to the public.

311. The State's COVID-19 emergency medical countermeasure program is so intimately regulated, licensed, and funded that "[t]he State has so far insinuated itself into a position of interdependence…that it must be recognized as a joint participant in the challenged activity" *Burton v. Wilmington Pkg. Auth*, 365 U.S. 715, 81 S. Ct. 856 (1961).

312. The State created a policy that required all medical facilities workers and healthcare workers to inject an investigational COVID-19 drug into their bodies to continue operations and or employment. Moreover, those who refused to comply were notified by the State that they could lose their license and/or face a $500 per day per violation fine. Therefore, the fine itself denotes

[63] Misuse of power, possessed by virtue of state law and made possible only because the wrongdoer is clothed with the authority of state law, is action taken "under color of" state law. *United States v. Classic*, 313 U.S. 299 (1941), citing *Ex parte Virginia,* 100 U. S. 339, 100 U. S. 346; *Home Telephone & Telegraph Co. v. Los Angeles,* 227 U. S. 278, 227 U. S. 287, *et seq.; Hague v. CIO,* 307 U. S. 496, 307 U. S. 507, 307 U. S. 519; *cf.* 101 F.2d 774, 790.

that the State assumes complete authority over all healthcare settings and workers relating to the CDC COVID-19 Vaccination Program.

313. "If a private actor is functioning as the government, that private actor becomes the state for purposes of state action."[64]

314. The State intimately relied on PeaceHealth to achieve its COVID-19 immunization goals.[65]

315. The federal government exclusively funds Oregon's COVID-19 Vaccination Program.

316. As the court held in *Modaber v. Culpeper Memorial Hospital, Inc.,* 674 F.2d 1023 (4tCir. 1982):

> "we must inquire 'whether there is a sufficiently close nexus between the State and the challenged action of the regulated entity that the action of the latter may fairly be treated as that of the State itself." *Jackson v. Metropolitan Edison Co.,* 419 U.S. 345, 351, 95 S.Ct. 449, 453, 42 L.Ed.2d 477 (1974); *accord, Flagg Brothers, Inc. v. Brooks,* 436 U.S. 149, 157, 98 S.Ct. 1729, 1733, 56 L.Ed.2d 185 (1978). In holding that a privately-owned utility's termination of service is not "state action", the Court in *Jackson* makes it clear that state involvement without state responsibility cannot establish this nexus. *See* 419 U.S. 358, 95 S.Ct. 457. A state becomes responsible for a private party's act if the private party acts (1) in an exclusively state capacity, (2) for the state's direct benefit, or (3) at the state's specific behest. It acts in an exclusively state capacity when it "exercises powers traditionally exclusively reserved to the state[,]" 419 U.S. 352, 95 S.Ct. 454; for the state's direct benefit when it shares the rewards and responsibilities of a private venture with the state, *see id.,* 357-58, 95 S.Ct. 456-57, *Burton v. Wilmington Parking Authority,* 365 U.S. 715, 723-24, 81 S.Ct. 856, 860-61, 6 L.Ed.2d 45 (1961); and at the state's specific behest when it does a particular act which the state has directed or encouraged."

[64] *Terry v. Adams*, 345 U.S. 461, 469-70, 73 S. Ct. 809, 97 L. Ed. 1152 (1953); *See Jackson v. Metropolitan Edison Co.*, 419 U.S. at 353, 95 S. Ct. 449.
[65] See Oregon State Provider FAQ attachment

317. PeaceHealth is under express COVID-19 vaccination protocols by the State and the CDC to include[66]:

A. Organization must administer COVID-19 Vaccine in accordance with all requirements and recommendations of CDC and CDC's Advisory Committee on Immunization Practices (ACIP),

B. Within 24 hours of administering a dose of COVID-19 Vaccine and adjuvant (if applicable), Organization must record in the vaccine recipient's record and report required information to the relevant state, local, or territorial public health authority,

C. Organization must submit Vaccine-Administration Data through either [a] the immunization information system (IIS) of the state and local or territorial jurisdiction or [b] another system designated by CDC according to CDC documentation and data requirements,

D. Organization must preserve the record for at least 3 years following vaccination, or longer if required by state, local, or territorial law,

E. Organization must not sell or seek reimbursement for COVID-19 Vaccine and any adjuvant, syringes, needles, or other constituent products and ancillary supplies that the federal government provides without cost to Organization,

F. Organization **must** administer COVID-19 Vaccine regardless of the vaccine recipient's ability to pay COVID-19 Vaccine administration fees, (emphasis added)

G. Before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) fact sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative,

H. Organization's COVID-19 vaccination services must be conducted in compliance with CDC's Guidance for Immunization Services During the COVID-19 Pandemic for safe delivery of vaccines,"

I. Organization must comply with CDC requirements for COVID-19 Vaccine management,

J. Organization must report the number of doses of COVID-19 Vaccine and adjuvants that were unused, spoiled, expired, or wasted as required by the relevant jurisdiction,

[66] See Exhibit A, CDC COVID-19 Vaccination Program Provider Requirements and Legal Agreement.

K. Organization must comply with all federal instructions and timelines for disposing COVID-19 vaccine and adjuvant, including unused doses,

L. Organization must report moderate and severe adverse events following vaccination to the Vaccine Adverse Event Reporting System (VAERS),

M. Organization must provide a completed COVID-19 vaccination record card to every COVID-19 Vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative. Each COVID-19 Vaccine shipment will include COVID-19 vaccination record cards,

N. Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine (emphasis added) and

O. Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws. (emphasis added).

318. The State of Oregon, via the CDC COVID-19 Vaccination Program Provider Agreement, must also ensure authorized agents of the State comply with 21 U.S.C. §360bbb-3's statutory requirements that the healthcare professional prospectively inform the recipient of:

A. the significant known and potential benefits and risks of the use of the product,

B. the option to accept or refuse administration of the product,

C. the health consequences of refusing the product, and,

D. the alternatives, risks, and potential benefits.

319. The State of Oregon and PeaceHealth must comply with the HHS Secretary's regulatory framework outlined in each EUA's Scope of Authorization[67], adding additional

[67] See "Authorizations of Emergency Use of Two Biological Products During the COVID-19 Pandemic; Availability," page 5208, Section "Emergency Stakeholders" continuing through "Vaccination Providers." https://www.federalregister.gov/documents/2021/01/19/2021-01022/authorizations-of-emergency-use-of-two-biological-products-during-the-covid-19-pandemic-availability

entanglement between PeaceHealth and the State. See *Brentwood Academy v. Tennessee Secondary School Athletic Assn*., 531 U.S. 288 (2001)

320. Therefore, "there is such a close nexus between the state and the challenged action that the seemingly private behavior may be fairly treated as that of the state itself." *Brentwood Academy v. Tennessee Secondary School Athletic Assoc*., 531 U.S. 288, 295 (2001). *See Jackson v. Metropolitan Edison Co*., 419 U.S. 345, 351, (1974).

321. In *Giron v. Corrections Corp. of America*, 14 F. Supp. 2d 1245 (D.N.M. 1998), the court stated, "If a state government must satisfy certain constitutional obligations when carrying out its functions, it cannot avoid those obligations and deprive individuals of their constitutionally protected rights by delegating governmental functions to the private sector. *See Terry v. Adams,* 345 U.S. 461, 73 S. Ct. 809, 97 L. Ed. 1152 (1953). The delegation of the function must carry with it a delegation of constitutional responsibilities."

322. The CDC Provider Agreement states, "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine (emphasis added)."

323. 21 U.S.C. §360bb-3 (a/k/a Section 564 of the FDCA, which authorizes EUA products under "applicable requirements" of the FDA) contains rights conferring language to accept or refuse EUA medical products without incurring a penalty or losing a benefit to which they are otherwise entitled.

324. Legally effective informed consent requires Defendants to ensure that they do not place individuals under a "sanction," "coercion," "undue influence," or "unjustifiable pressures" when offering participation in the use of an investigational medical product.

325. The COVID-19 investigational drugs are federal property and may not be administered to persons without first obtaining their legally effective informed consent to ensure the government's legal obligations under 45 CFR 46 (specifically §§ 116, 122) are met.

326. The CDC required Defendants to obtain the legally effective informed consent of individuals on its behalf when offering COVID-19 investigational new drugs via its Vaccination Program Provider Agreement.[68]

327. The CDC informed Defendants that "before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) Fact Sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative.[69]" The Fact Sheet serves as a function of obtaining Plaintiffs' legally effective informed consent.

328. Therefore, the federal government choosing to purchase all COVID-19 drugs under 21 U.S.C. §360bb-3 authority is Constitutionally obligated to obtain the legally effective informed consent of individuals prospectively and equally. The State, by extension, is also required to comply with the federal requirement and may not delegate this function to a private party without also delegating the Constitutional obligation.

329. Obtaining the individual's legally effective informed consent is a legal procedure, and the State cannot delegate the COVID-19 immunization program without empowering the private party with legal authority to obtain Plaintiffs' legally effective informed consent on the State's behalf.

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[68] Number 12(a) of the Agreement states "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine." This section requires adherence to 21 U.S.C. §360bbb-3 (Section 564) protocols.

[69] Each EUA letter mandates that the manufacturer will provide the drug's Fact Sheet to healthcare providers and that healthcare providers "will provide the Fact Sheet for Recipients and Caregivers to each individual receiving vaccination." Accordingly, the CDC COVID-19 Vaccination Program Provider Agreement has the same requirement.

330. Therefore, the Constitutional duty placed upon the State is to ensure that all persons choosing to accept or refuse are treated equally before the law by providing that neither "option" is penalized. Moreover, if a person is not treated equally before the law, then that unequal treatment must be conditioned upon due process.

331. PeaceHealth previously assured the Health and Human Services' Office of Human Research Protections that they would never violate the ethical principles outlined in the Belmont Report or their obligations under 45 CFR 46. In return for that assurance, HHS awarded them the right to participate in federal funding by providing them with the Federal Wide Assurance Agreement compliance number FWA00003906.

332. Oregon Health Authority also assured HHS and accordingly received the federal agreement number FWA00000520 signifying a legally binding agreement between Oregon and the federal government.

333. Defendants had a Constitutional duty under the Fourteenth Amendment to ensure persons accepting or refusing drugs, biologics, or devices falling under the authority of (1) 21 U.S.C §360bbb-3, (2) Article VII ICCPR Treaty, (3) Oregon Health Authority FWA00000520, (4) 45 CFR 46, (5) Prep Act, and (6) the CDC COVID-19 Vaccination Program Provider Agreement, were not deprived of their equal protection rights or liberty or property without due process when exercising either option.

334. As the U.S. Supreme Court stated in *Mullane v. Central Hanover Bank & Trust Co*., 339 U.S. 306 (1950):

> Many controversies have raged about the cryptic and abstract words of the Due Process Clause, but there can be no doubt that, at a minimum, they require that deprivation of life, liberty or property by adjudication be preceded by notice and opportunity for hearing appropriate to the nature of the case.
>
> * * *

> An elementary and fundamental requirement of due process in any proceeding which is to be accorded finality is notice reasonably calculated, under all the circumstances, to apprise interested parties of the pendency of the action and afford them an opportunity to present their objections. *Milliken v. Meyer,* 311 U. S. 457; *Grannis v. Ordean,* 234 U. S. 385; *Priest v. Las Vegas,* 232 U. S. 604; *Roller v. Holly,* 176 U. S. 398. The notice must be of such nature as reasonably to convey the required information, *Grannis v. Ordean, supra,* and it must afford a reasonable time for those interested to make their appearance, *Roller v. Holly, supra, and cf. Goodrich v. Ferris,* 214 U. S. 71.

335. In August 2021, Defendants informed individuals under their authority that they require mandatory COVID-19 investigational drug use of all healthcare workers, contractors, and other employees and set mandatory timelines for compliance.

336. At no point before, during, or after the State of Oregon and PeaceHealth issued their mandatory investigational drug policies were Defendants in possession of COVID-19 drugs licensed by the FDA and also introduced into commerce for general commercial marketing having a legal indication to immunize Plaintiffs from the SARS-CoV-2 (COVID-19) infection. (See discussion *infra*)

337. Defendants informed Plaintiffs of pending action that would rob them of their liberty (e.g., restrictions of facility use) and property (e.g., wages, insurance, paid-time-off, career) should they exercise their federally secured rights to refuse Defendants' unlawful demands. Defendants did not provide Plaintiffs with an opportunity to be heard or the right to defend their federally secured rights when depriving them of their liberties and property. *See Louisville and Nashville R. R. Co. v. Schmidt*, 177 U.S. 230, 236, 20 S.Ct. 620, 44 L.Ed. 747 (1900).

338. Defendants refused to acknowledge federal laws providing Plaintiffs with the explicit authority to refuse COVID-19 investigational drugs without incurring a penalty or losing a benefit to which they were otherwise entitled.

339. **If persons in authority, such as Defendants, refuse to acknowledge rights conferred upon Plaintiffs by valid acts of Congress, then due process is legally impossible to secure.**

340. Therefore, PeaceHealth, acting under color of law, violated Plaintiffs' substantive and procedural due process rights under the 14th Amendment.

341. The Ninth Circuit reminded us in *Rawson v. Recovery Innovations, Inc.*, No. 19-35520 (9th Cir. 2020) that "The Supreme Court has … held that private parties may act under color of state law when they perform actions under which the state owes Constitutional obligations to those affected."

342. 21 U.S.C. §360bbb-3 (Section 564) creates an express right for individuals to accept or refuse the administration of products not licensed by the FDA for general commercial marketing during a declared emergency.

343. 45 CFR 46, the Belmont Report, Article VII of the ICCPR Treaty, and the FWA all require the individual's legally effective informed consent before administering an investigational medical product.

344. Congress mandated that should a person accept or refuse to participate in the use of an investigational medical product, no penalty or loss of benefits could be imputed to them.

345. Congress expressly preempts state laws conflicting with the PREP Act.

346. Congress preempts state laws conflicting with 21 U.S.C. §360bbb-3.

347. Therefore, the State is bound by the Supremacy Clause to comply with the obligations above when delegating the COVID-19 immunization program, which relies exclusively on COVID-19 investigational drugs.

348. Therefore, Defendants had a Fourteenth Amendment constitutional obligation and responsibility to ensure all persons were under the equal protection of laws regarding the CDC COVID-19 Vaccination Program,[70] PREP Act, 21 U.S.C. §360bbb-3 protocols, or a medical countermeasure's Scope of Authorization.

349. No person who *refuses* an EUA medical product issued under the PREP Act authority can be treated differently before the law than a person who *accepts* an EUA medical product issued under the PREP Act authority.

350. No person choosing the 21 U.S.C. §360bbb-3 option to *refuse* can be treated differently before the law than a person choosing the option to *accept*.

351. Therefore, the public function of administrating the CDC COVID-19 Vaccination program places a duty upon Defendants to protect the due process and equal protection rights of individuals participating in that public function.

352. Given the enormity of federal laws, regulations, and contracts, Defendants effectually deprived Plaintiffs of their Constitutional protections and statutory rights when acting under the color of law.

353. PeaceHealth, among other Oregon healthcare facilities, acted under the color of a State custom[71] and usage to deprive Plaintiffs of their 14th Amendment constitutional protections.

354. The State's custom was to ignore the statutory rights of individuals to refuse the administration of an EUA product without incurring a penalty or losing a benefit to which they were otherwise entitled. The custom of the State was so pervasive that it relegated citizens who

[70] The CDC COVID-19 Vaccination Program relied exclusively on investigational medical products that were also under the statutes mentioned above. Therefore, the 14th Amendment violations related to federal statutes and not exclusively to the CDC COVID-19 Vaccination Program.

[71] 18 U.S.C. §1983: "Every person who, under color of any statute, ordinance, regulation, custom, or usage, of any State…" (Emphasis added).

exercised their federal right to refuse to that of second-class citizens by refusing to protect their equal protection rights.

355. The Oregon Health Authority refused to issue a public notice that the Plaintiffs had the express right to refuse EUA products without consequence and refused to enforce that right when healthcare facilities and workers abridged that right.

356. Oregon State Attorney General Ellen F. Rosenblum also refused to protect the rights of Oregon's citizens through enforcement when the Governor, state agencies, and PeaceHealth violated the law.

357. The Supremacy Clause dictates that 21 U.S.C. §360bbb-3 is the supreme law of the land within the State of Oregon. Therefore, an abridgment of 21 U.S.C. §360bbb-3 enabled State Executive officers' authority to correct the errant behavior due to their FWA, IRBs, and CDC COVID-19 Vaccination agreement.

358. Nearly 67,000 nurses in Oregon[72] were denied their legally effective informed consent rights, and thousands were deprived of their ability to be employed, all without consequence by Attorney General Ellen F. Rosenblum to healthcare facilities that engaged in that unlawful activity.

359. The Governor's Proclamation reinforced the unlawful state custom that a person within the boundaries of Oregon cannot act on their statutory right to refuse an investigational drug but must seek an exemption only under reasons of "disability," "medical," or a "sincerely held religious belief." (See #5 of E.O. 21-29)

360. Patrick Allen also reinforced the illegal custom by stating in OHA's PH 38-2021 Temporary Administrative Order, "On or before October 18, 2021, healthcare providers and

[72] Authority O. Accessed August 13, 2023. https://www.oregon.gov/oha/HPA/HP-HCW/Meeting%20Documents/5.-Health-Occupations-Report.pdf

healthcare staff must provide their employer, contractor, or responsible party with either: Proof of vaccination…or Documentation of a medical or religious exemption."

361. To require an individual to request an exemption from investigational drug use is a violation of federal law and the duties of Defendants under the CDC COVID-19 Vaccination Program Provider agreement, FWA agreement, and 45 CFR 46 regulations. Defendants are not empowered to amend federal legislation or to require that which Congress prohibits.

362. The Supreme Court noted in *Adickes v. S. H. Kress & Co*., 398 U.S. 144 (1970) that the "Petitioner will have established a claim under § 1983 for violation of her equal protection rights if she proves that she was refused service by respondent because of a state-enforced custom…" (emphasis added)

363. Plaintiffs were denied access to living wages and demoted to second-class citizenry because of a State-enforced custom that Defendants incorrectly asserted had a force of law overriding the authority of the United States Constitution.

364. The State custom resulted in the Defendants ignoring the authority of Congress by amending 21 U.S.C. § 360bbb-3 as follows:

A. Congress only authorized the Secretary to grant access to experimental medical products and the conditions under which that access can occur. Defendants illegally established conditions contrary to those established by the Secretary,

B. Congress only authorized the Secretary to determine who can participate in any activity that becomes lawful to 21 U.S.C. § 360bbb-3. However, the Secretary is prohibited from requiring any person to participate in any such activity. Yet, Defendants usurped the Constitutional authority of Congress and mandated that all healthcare facilities and providers must participate in the EUA program,

C. Congress expressly preempted all state laws via the PREP Act and 21 U.S.C. § 360bbb-3. Yet, Defendants usurped the authority of Congress, ignored the Constitution's Supremacy Clause, and

> enforced laws, customs, ordinances, and regulations in defiance of Congress, the Constitution, and the rights of Plaintiffs.

365. Defendants' actions demonstrate a willful, intentional, and wanton disregard for the United States Constitution, federal law, agency regulations, and the express rights of individuals to determine EUA participation as established by a valid act of Congress.

366. Moreover, the Court held, "Based upon the language of the statute legislative history [sic], and judicial decisions, the words 'under color of a . . . custom or usage, of [a] State,' in § 1983, mean that the 'custom or usage' must have the force of law by virtue of the persistent practices of state officials. Pp. 398 U. S. 162-169."

367. Although *Adickes* involved the state custom of racial discrimination, the precise custom is not the relevant point but rather the pervasiveness and persistence of the custom or usage, no matter its name.

368. The *Adickes* Court referenced one of the original Congressional supporters of § 1983, who stated, "[T]he chief complaint is not that the laws of the State are unequal, but that, even where the laws are just and equal on their face, yet, by a systematic maladministration of them, or a neglect or refusal to enforce their provisions, a portion of the people are denied equal protection under them."

369. The Court commented, "This interpretation of custom recognizes that settled practices of state officials may, by imposing sanctions or withholding benefits, transform private predilections into compulsory rules of behavior no less than legislative pronouncements."[73]

370. Acting under State custom, Defendants usurped the federal government's authority, imposed sanctions, and withheld benefits as if that fraudulent authority had legislative pronouncement.

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[73] shouldnt we put like ID etc here? I dunno but you do :)

371. After PeaceHealth deprived Plaintiffs of their Fourteenth Amendment rights to due process and altered their employment, the State of Oregon Employment Department enforced the custom by denying some Plaintiffs unemployment benefits solely based on them exercising their 21 U.S.C. §360bb-3 legal rights.

372. The State Employment Department provided a FAQ of COVID-19-related questions. The FAQ stated, "in general, if your employer has a policy requiring COVID-19 vaccinations, and you are unwilling to get the vaccine or use a reasonable alternative protection if your employer permits it, your claim may be denied," and "You will generally not be denied benefits for refusing to get the vaccine if you have a sincerely held religious belief, are medically unable to get the vaccine, or if a collective bargaining agreement or law exempts you from the vaccination requirement."[74]

373. The State Employment Department did not provide an answer to the question of whether or not it was lawful for an employer to require an employee to participate in the use of an investigational drug if they did not fall into one of those three categories (religious, medical, union). The State agency treated currently available drugs as though an employer could mandate their use under threat of penalty.

374. The State's Employment Security Department violated the Unconstitutional Conditions Doctrine by enforcing a custom as though it had the force of law by requiring Plaintiffs to surrender their constitutional and statutory rights as a condition for accessing public benefits. The violation of the Unconstitutional Conditions Doctrine is demonstrated by the fact that the Employment Security Department's laws were "just and equal on their face." Still, it relied on the

[74] COVID-19 Vaccination. Oregon Employment Department. Published April 12, 2022. Accessed August 13, 2023. https://oedcontactus.oregon.gov/hc/en-us/articles/4939841547287-COVID-19-Vaccination

use of investigational medical products for determining unemployment benefits, causing a severe violation of Plaintiffs equal protection and due process rights.

375. PeaceHealth and Oregon State Employment Security Department defamed the character and reputation of Plaintiffs who were denied unemployment benefits by labeling their exercising of a federal right as misconduct.

376. The COVID-19 program did not belong to PeaceHealth. PeaceHealth was only allowed to provide the program as a public function on behalf of the State. PeaceHealth unlawfully usurped the federal government's authority and required that which Congress prohibits because Defendants developed a custom to which they endowed more authority than federal law.

377. OHA and Director Patrick Allen, having its own FWA agreement and intimate knowledge of the Belmont Report and 45 CFR 46 requirements, refused to enforce the laws on the books and allowed facilities and persons it licenses to subject individuals to investigational drug use under threat of incurring a penalty or losing a benefit to which they were otherwise entitled.

378. OHA, after authorizing numerous entities to enroll in the CDC COVID-19 Vaccination Program and PREP Act activities, refused to enforce the provisions of the CDC Vaccination Program Provider Agreement when those entities violated the terms and conditions of the contract and its supporting statutes, mainly 21 U.S.C §360bb-3. OHA held the authority to intervene but chose to act on a non-enforcement custom to support Governor Brown's unlawful edicts.

379. The Oregon Medical Board, a State agency mandated with regulating and enforcing the quality standards of medicine, refused to correct the errant behavior of licensed healthcare professionals mandating individuals to participate in the use of 21 U.S.C. §360bbb-3 investigational drugs.

380. Municipalities across the State also required citizens to participate in investigational COVID-19 drugs as a condition to enjoy public employment, access to facilities, and other benefits in violation of federal law and the citizens' rights, all without consequence by any law enforcement agency within the State or local municipality.

381. The violations of the Constitutional and statutory rights of Oregon citizens are so pervasive in the State that an average person could only conclude that the United States Constitution no longer has the force of law within its boundaries.

382. By and through the above-described facts and law, the Plaintiffs have established that a state-enforced custom abridged their federally secured rights to refuse an investigational drug without penalty.

## XX. The Spending Clause

383. As stated in *Gonzaga Univ. v. Doe*, 536 U.S. 273 (2002), "the Court has found that spending legislation gave rise to rights enforceable under § 1983 only in *Wright* v. *Roanoke Redevelopment and Housing Authority,* 479 U. S. 418, 426, 432, and *Wilder* v. *Virginia Hospital Assn.,* 496 U. S. 498, 522523, where statutory provisions explicitly conferred specific monetary entitlements upon the plaintiffs, and there was no sufficient administrative means of enforcing the requirements against defendants that failed to comply." See also, *Health and Hospital Corporation of Marion County v. Talevski*, *supra*, 599 U.S. ____ (2023)

384. The federal government funds all COVID-19 EUA shots via Medicare.[75]

385. The executive branch of government established the CDC COVID-19 Vaccination Program Provider Agreement to execute the government's objective.

[75] https://www.medicare.gov/medicare-coronavirus

386. Only persons authorized to participate in the CDC Vaccination Program can bill the government for administered shots.

387. The State must authorize all persons to participate in the CDC Vaccination Program and Oregon established the guidelines by which healthcare settings could administer the federal government's fully-funded property.

388. The Spending Agreement lacks any enforcement scheme that would preclude § 1983 enforcement.

389. Requirement Number 3 of the CDC Provider Agreement states, "Organization must not sell or seek reimbursement for COVID-19 Vaccine and any adjuvant, syringes, needles, or other constituent products and ancillary supplies that the federal government provides without cost to Organization." (See Exhibit A.)

390. Requirement Number 4 of the CDC Provider Agreement states, "Organization must administer COVID-19 Vaccine regardless of the vaccine recipient's ability to pay COVID-19 Vaccine administration fees."

391. Those two provisions establish a specific monetary entitlement to the individual.

392. Requirement Number 5 of the CDC Provider Agreement states, "Before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) Fact Sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative."

393. Requirement Number 5 complies with funding restrictions established by Congress in 45 CFR §122 and 10 U.S.C. §980.

394. The compliance is found in the EUA Fact Sheet, notating the individual's right to accept or refuse the administration of the product with no attached conditions. This express right

is the fundamental requirement in obtaining the legally effective informed consent of the individual.

395. Whether for civilians under 45 CFR § 46.122 or military personnel under 10 U.S.C. §980, Congress created a specific monetary entitlement for individuals considering whether or not to participate in a federally funded research activity. That entitlement means they have the explicit right to be informed of the risks, benefits, and alternatives to the research product and then consider whether to participate without incurring a fee or being under outside pressure to participate.

396. This monetary entitlement is most apparent in the CDC COVID-19 Vaccination Program Provider Agreement. An individual can seek out a participating COVID-19 Program healthcare professional, obtain medical counseling, ask questions, and read literature. If they choose not to participate, they will not incur a fee from the professional for the administrative time spent considering whether or not to participate since the healthcare professional must inform them of their legal right to refuse under 21 U.S.C. §360bbb-3.

397. The healthcare professional administering the EUA product agreed to comply with the legally effective consent requirements via Agreement Number 12 on the CDC COVID-19 Vaccination Program Provider Agreement mandating that (1) "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine," and (2) "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws."

398. The "all applicable requirements as set forth by the U.S. Food and Drug Administration, including…any EUA" extends to 21 USC 360bbb-3 (Section 564), 45 CFR 46,

the FWA, the IRB, the ICCPR Treaty, and the Scope of Authorization letter for each respective EUA.

399. Therefore, 21 U.S.C. §360bbb-3, 45 CFR § 46.122, and 10 U.S.C. §980 clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983 when federal funds are expended under those provisions of law.

**XXI. Deprivations of Constitutional and Statutory Rights, Privileges, and Immunities**

400. OHA's statement (PH 38-2021) that "On or before October 18, 2021, healthcare providers and healthcare staff must provide their employer, contractor or responsible party with either: • Proof of vaccination showing they are fully vaccinated; or • Documentation of a medical or religious exception" was no less than a direct attempt to subvert the Constitution of the United States of America.

401. Defendants were well aware that 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) contained a required condition of Congress that they must "ensure that individuals to whom the product is administered are informed — of the option to accept or refuse administration of the product."

402. The word "option" denotes a legal right held by the individual considering participation in which Defendants are prohibited by law from interfering by subjecting individuals to punitive acts based on their chosen option.

403. Defendants were well aware that 42 USC 247d-6d(b)(8) preempted their authority to establish conditions that would otherwise conflict with the federal government's voluntary medical countermeasure program and 21 U.S.C. §360bbb-3 expanded access emergency use authorizations. Moreover, Defendants knew that the Supremacy Clause prevented them from establishing conditions contrary to 21 U.S.C. §360bbb-3.

404. Defendants were well aware that their voluntary agreement to abide by the terms and conditions of the CDC COVID-19 Vaccination Program required them to obtain the legally effective informed consent of Plaintiffs equally and in advance of the product's administration. That legal requirement meant that, at minimum, Defendants had to establish conditions ensuring potential participants were not under outside pressure to participate in any 21 U.S.C. §360bbb-3 product or 42 USC 247d-6d(b)(8) activity.

405. However, Defendants proceeded as if Plaintiffs' legal right to accept or refuse participation in activities or the use of products under the above-mentioned statutory authorizations were conditional, not absolute. Such nonsensical behavior by State actors shocks the conscience. Nothing in 21 U.S.C. §360bbb-3 or 42 USC 247d-6d(b)(8) could lead any person to believe that any State authority has the right to interfere in the decision-making process of an individual considering participation in the use of a 21 U.S.C. §360bbb-3 medical countermeasure. That is why Defendants concealed the Plaintiffs' legal rights under 21 U.S.C. §360bbb-3 in all of their proclamations.

406. Congress expressly preempted Defendants from establishing conditions that would otherwise conflict with the federal government's CDC COVID-19 Vaccination Program under 42 USC 247d-6d(b)(8) and 21 U.S.C. §360bbb-3.

407. Therefore, when Defendants, acting under color of law, enacted laws, regulations, ordinances, and customs requiring Plaintiffs to become "fully vaccinated" before the availability of licensed vaccines, they willfully usurped the authority of the HHS Secretary, ignored the authority of Congress, and intentionally robbed Plaintiffs of their Constitutional protections guaranteed to them under the Fourteenth Amendment.

408. Defendants treated persons exercising the federally secured right to accept a COVID-19 investigational product (drugs, testing articles, masks) differently than persons exercising the federally secured right to refuse the product. A valid Act of Congress enacted both options, which must be treated equally before the law. Defendants established punitive conditions only on persons choosing the option to refuse, violating the 14th Amendment's Equal Protection Doctrine.

409. Defendants, at all times pertinent, engaged in moral turpitude by pretending as if Plaintiffs did not have the legal authority to refuse a 21 U.S.C. §360bbb-3 product (drugs, biologics, devices) without consequence. Therefore, irrespective of any established procedures by Defendants to allow Plaintiffs to air their concerns about the COVID-19 vaccination requirements, Due Process was impossible to secure because Defendants intentionally acted on a custom ignoring Plaintiffs' statutory rights and Congress of its Constitutional authority.

410. Before Defendants could legally access and distribute any COVID-19 investigational drug, they had to agree to comply with the terms and conditions established by the executive branch of the federal government as outlined in the CDC COVID-19 Vaccination Program Provider Agreement. That agreement required Defendants to comply with the Scope of Authorization for each EUA and the laws under 21 U.S.C. §360bbb-3, among others.

411. The CDC COVID-19 Vaccination Program Provider Agreement provides the following statutory and third-party beneficiary rights[76] to Plaintiffs:

A. to give legally effective informed consent,

B. to consider participation without fearing consequences based on the chosen option,

[76] The CDC COVID-19 Vaccination Program Provider Agreement states in plain language at 12(a) that "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine." The listed items are associated with the "applicable" laws related to this complaint.

C. having all associated costs and fees paid for by the federal government irrespective of the chosen option,

D. to be informed of the risks/benefits/alternatives to the medical countermeasure,

E. to cease participation at any time without consequences,

F. to be treated with dignity when discussing the option with a healthcare provider, as discussed in detail in the Belmont Report.

412. Although Defendants voluntarily agreed to comply with the terms and conditions of the CDC COVID-19 Vaccination Program, they at all times pertinent violated the third-party beneficiary rights outlined in the Contract.

413. 45 CFR § 46.116 contains the only known statutory definition of legally effective informed consent.[77] Therefore, when Congress explicitly or implicitly mandates that a person give their informed consent when involved in the use of an investigational drug, this regulation becomes the only source courts can utilize to ascertain its meaning.

414. 21 U.S.C. §360bbb-3 protocols do not require written informed consent. However, the conditions necessary to establish legally effective informed consent remain the same irrespective of written or oral informed consent requirements, mainly that there can be no outside pressure on Plaintiffs to use a drug not licensed by the FDA for general commercial marketing. Defendants violated the legally effective informed consent doctrine when applying coercion, undue influence, unjustifiable pressures, sanctions, and other punitive acts to Plaintiffs when they did not achieve "fully vaccinated" status by the compliance date. This achievement could only be

[77] Congress specifically requested the HHS Secretary to promulgate regulations pertaining to the protection of human subjects via the 1974 National Research Act. Therefore, these regulations represent the express intent of Congress when discussing what is legally effective informed consent.

obtained by relying exclusively on 21 CFR 312.3 investigational new drugs under research conditions.

415. At all times pertinent, Defendants were under strict requirements via the CDC COVID-19 Vaccination Provider Agreement, 21 U.S.C. §360bbb-3, 45 CFR 46.101(c), and 45 CFR 46.101(i) to comply with the ethical principles of the Belmont Report holding third-party beneficiary rights for Plaintiffs.

416. The Belmont Report informed Defendants that:

> A. "Respect for persons incorporates at least two ethical convictions: first, that individuals should be treated as autonomous agents, and second, that persons with diminished autonomy are entitled to protection. The principle of respect for persons thus divides into two separate moral requirements: the requirement to acknowledge autonomy and the requirement to protect those with diminished autonomy,"
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> B. "An autonomous person is an individual capable of deliberation about personal goals and of acting under the direction of such deliberation. To respect autonomy is to give weight to autonomous persons' considered opinions and choices while refraining from obstructing their actions,"
>
> C. "To show lack of respect for an autonomous agent is to repudiate that person's considered judgments, to deny an individual the freedom to act on those considered judgments."

417. Defendants failed in their duties to respect the autonomy of Plaintiffs and refused to respect their considered choices, and most certainly obstructed Plaintiffs' actions to make an autonomous choice.

418. The Belmont Report further instructed Defendants that "Persons are treated in an ethical manner not only by respecting their decisions and protecting them from harm, but also by making efforts to secure their well-being. Such treatment falls under the principle of beneficence. The term 'beneficence' is often understood to cover acts of kindness or charity that go beyond strict obligation. In this document, beneficence is understood in a stronger sense, as an obligation.

Two general rules have been formulated as complementary expressions of beneficent actions in this sense: (1) do not harm and (2) maximize possible benefits and minimize possible harms."

419. 21 U.S.C. §360bbb-3 requires the healthcare professional and the recipient to mutually agree to the product's administration before the process becomes legal. This legal requirement by Congress is to ensure that proper medical counseling protects individuals from potentially life-altering adverse effects from using a medical countermeasure product.

420. Defendants intentionally withheld factual knowledge that the EUA mRNA products were causing historical harm to recipients. Moreover, there was no historical knowledge or means of ascertaining the more than 6.859 trillion[78] potential contraindications the products could have had with existing medical conditions and other drugs. That is why drugs undergo years of trials and testing before receiving their marketing approval. The lack of quality testing and fear of liability is also why, as of this filing, no COVID-19 drug manufacturer has shipped their licensed products in violation of 21 CFR 312.7(c).[79]

421. The Defendants' actions demonstrate a wanton disregard for Plaintiffs' safety and health. Moreover, Defendants' actions demonstrate a violation of the Belmont Report to "minimize possible harms."

422. The Belmont Report was explicit in Defendants' ethical requirement on how to legally and effectively obtain informed consent as follows:

> A. "Respect for persons requires that subjects, to the degree that they are capable, be given the opportunity to choose what shall or shall not happen to them. This opportunity is provided when adequate standards for informed consent are satisfied (see, supra),"

[78] The FDA has licensed 19,000 marketable drugs, and in 2021, three mRNA drugs were available to Plaintiffs. Therefore, 19,000 to the power of three equals 6,859,000,000,000 potential contraindications that mere months of research could not have exposed.

[79] 21 CFR 312.7(c) provides: "*Prolonging an investigation*. A sponsor shall not unduly prolong an investigation after finding that the results of the investigation appear to establish sufficient data to support a marketing application."

B. "An agreement to participate in research constitutes a valid consent only if voluntarily given."

423. "Voluntarily given" has no other meaning than the action was free from coercion, unjustifiable pressures, undue influence, and not given out of fearing consequences. Defendants intentionally violated their duties to obtain the Plaintiffs' legally effective informed consent, thus violating the ethical principles of the Belmont Report and its intended benefits for the Plaintiffs.

424. The State of Oregon and PeaceHealth provided assurances (FWA agreement) to HHS as a condition to receive federal funds to involve humans in the use and evaluation of investigational medical products. At a minimum, those assurances must comply with 45 CFR 46 protocols and the ethical principles of the Belmont Report.

425. The assurances aim to benefit third parties considering using investigational products offered by Defendants for various research-related purposes. Factually speaking, the only purpose of the FWA agreement is to benefit non-signatory third parties. Defendants (promisors) assured the federal government (promisee) that they would protect the rights, safety, and health of Plaintiffs (non-signatory party) by promising to abide by the protective protocols under 45 CFR 46 and the Belmont Report; protocols having an unambiguous focus on the third-party beneficiary (Plaintiffs). Defendants' promise to the federal government was by direct nexus also a promise to any third-party participant, such as Plaintiffs.

426. The duties imposed upon Defendants is to obtain legally effective informed consent from Plaintiffs before the product is administered. When Defendants imposed sanctions, such as termination from employment and denial of access to living wages, they violated the intended protective benefits Congress conferred upon Plaintiffs via the Federal Wide Assurance agreement. Defendants voluntarily agreed to comply with their FWA obligations, but Defendants voluntarily violated that agreement harming the intended benefits of Plaintiffs under the FWA agreement.

427. 45 CFR § 46.116 was written by the express consent of Congress as instructed under the National Research Act (1974) (see, *supra*). The entire purpose of this provision of law is to provide individuals involved in federally funded medical research projects with protective legal benefits. The section of the law is unambiguous in its right conferring language, which focuses exclusively on the third-party beneficiary. Some of those intended benefits are:

> A. "An investigator shall seek informed consent only under circumstances that provide the prospective subject or the legally authorized representative sufficient opportunity to discuss and consider whether or not to participate and that minimize the possibility of coercion or undue influence,"
>
> B. "The information that is given to the subject or the legally authorized representative shall be in language understandable,"
>
> C. "The prospective subject or the legally authorized representative must be provided with the information that a reasonable person would want to have in order to make an informed decision about whether to participate, and an opportunity to discuss that information,"
>
> D. "No informed consent may include any exculpatory language through which the subject or the legally authorized representative is made to waive or appear to waive any of the subject's legal rights, or releases or appears to release the investigator, the sponsor, the institution, or its agents from liability for negligence,"
>
> E. "A statement [must be given to the recipient] that participation is voluntary, refusal to participate will involve no penalty or loss of benefits to which the subject is otherwise entitled, and the subject may discontinue participation at any time without penalty or loss of benefits to which the subject is otherwise entitled," and
>
> F. "A statement that the particular treatment or procedure may involve risks to the subject."

428. Defendants voluntarily participated in COVID-19 federal funding to conduct research activities on its behalf involving humans. Defendants intentionally ignored their duties to third-party beneficiaries (Plaintiffs) under 45 CFR § 46.116 when establishing employment conditions on receiving one of the 21 U.S.C. §360bbb-3 medical countermeasure products in direct

violation of federal law and their legal obligations under the FWA and the CDC COVID-19 Vaccination Program Provider Agreement. Defendants, at all times pertinent, were well aware of the rights of Plaintiffs to refuse investigational medical products, irrespective of their authorized expanded access protocol, without consequence but chose to engage in moral turpitude for the express purpose of causing Plaintiffs to surrender their Constitutional protections and statutory rights.

429. Article VII of the ICCPR Treaty binds Defendants, as State Actors, to comply with its principles and legal obligations. In the same legal manner as the Belmont Report, 45 CFR 46, 21 U.S.C. §360bbb-3, FWA, the PREP Act, and the CDC COVID-19 Vaccination Program Provider Agreement, Article VII denies Defendants the legal authority to subject Plaintiffs to medical experimentation outside of their free will and voluntary consent. Although Defendants were unsuccessful in causing Plaintiffs to surrender their Constitutional and statutory rights, Defendants subjected Plaintiffs to medical experimentation when issuing their respective COVID-19 vaccination requirements and then penalizing them for not complying with those unlawful requirements. Defendants intentionally failed to fulfill their duties under Article VII of the ICCPR Treaty.

430. The PREP Act (42 USC 247d-6d,6e) expressly strips Defendants of all legal authority to amend the voluntary nature of the medical countermeasure program by enacting laws, regulations, and customs that would otherwise allow Defendants to interfere with the objectives of the federal government issuing the declared emergency.

431. Defendants, ignoring Congressional authority, established laws, regulations, and customs that interfered with the PREP Act and 21 U.S.C. §360bbb-3 protocols when they issued their respective COVID-19 vaccination requirements.

432. PeaceHealth, acting under the color of law, issued its COVID-19 mandate on August 30, 2021 stating in part, (1) "It is the policy of PeaceHealth to require healthcare workers (HCWs) to be fully vaccinated against COVID-19," and (2) "The compliance date for all HCWs to start a COVID-19 vaccination series/dose or submit for an exemption will be August 31, 2021. The compliance date for all HCWs to be fully vaccinated or have an approved medical or religious exemption that can be accommodated is October 15, 2021."

433. PeaceHealth, CEO Liz Dunne, Todd Salnas, and CPE Doug Koekkoek are legally sophisticated and have a history of conducting joint investigational trials with the National Cancer Institute.[80] Moreover, PeaceHealth's medical professionals have administered investigational drugs under 21 U.S.C. §360bbb et seq. protocols to individuals for years before the pandemic. They know of the statutes' requirements when administering investigational medical products authorized under the statute's provisions.

434. Therefore, when Governor Brown's Oregon Health Authority issued its proclamation ordering healthcare facilities to subject individuals under their authority to investigational drug use, PeaceHealth knew that the Executive Proclamation was a direct violation of federal law and their lawful duties under the CDC Vaccination Program Provider Agreement.

435. PeaceHealth, like all other healthcare facilities in the State, was free to comply with laws and regulations protecting humans involved in investigational medical products.

436. However, due to the customs of the State of Oregon, where certain laws are not enforced by its Attorney General, as demonstrated by the Governor's willful violations of federal law, PeaceHealth chose to hide behind the State's mandate and customs to participate in the federal

[80] https://www.peacehealth.org/research-and-clinical-trials

government's COVID-19 cash cow at the expense of Plaintiff's 14th Amendment and statutory rights.

437. PeaceHealth's COVID-19 vaccine policy relied exclusively on investigational new drugs not classified by the FDA as a vaccine.

438. PeaceHealth never informed the Plaintiffs of their right to refuse an investigational new drug without consequence, as required by federal law.

439. PeaceHealth never set a time, date, or place to secure Plaintiff's due process rights.

440. PeaceHealth, CEO Liz Dunne, Todd Salnas, and CPE Doug Koekkoek violated Plaintiffs' 14th Amendment rights when penalizing them for refusing the administration of a COVID-19 investigational drug by October 15, 2021.

441. PeaceHealth only altered the employment of those who refused an EUA product and not those who accepted, severely violating the 14th Amendment's Equal Protection Clause.

442. PeaceHealth only terminated healthcare insurance for Plaintiffs placed on unpaid administrative leave after they refused administration of an EUA drug or refused participation in a PREP Act activity, which is a deprivation of the Plaintiffs' 14th Amendment rights of equal protection.

443. PeaceHealth only required N95 masks (also under PREP Act authority) for some of the Plaintiffs after they refused administration of an EUA drug or refused participation in a PREP Act activity, which is a deprivation of the Plaintiffs' 14th Amendment rights of equal protection.

444. PeaceHealth only required COVID-19 testing (i.e., utilizing EUA products), for some of the Plaintiffs after they refused administration of an EUA drug or refused participation in a PREP Act activity, which is a deprivation of the Plaintiffs' 14th Amendment rights of equal protection.

445. PeaceHealth did not afford Plaintiffs a date, time, or place to exercise their due process rights. Therefore, PeaceHealth, as State Actors violated Plaintiffs' substantive and procedural due process rights.

446. PeaceHealth refused, at all times, to obtain the legally effective informed consent of Plaintiffs, thereby violating Plaintiffs' rights under 45 CFR 46, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, the Belmont Report, and PeaceHealth duties under its Federal Wide Assurance agreement, Institutional Review Boards, and its signed CDC COVID-19 Vaccination Program Provider Agreement.

447. At all times pertinent herein, Plaintiffs were legally and physically incapable of violating any State or PeaceHealth requirement to receive a licensed COVID-19 vaccine because no such vaccine existed by which they could refuse. This unassailable fact did little to stop the tyrannical dictates of Defendants from punishing Plaintiffs when they exercised their lawful statutory authority.

## XXII. Wanton Disregard for the Safety, Health, and Rights of Plaintiffs

448. Governor Brown's statement (Executive Order 21-29) that, "the arrival of safe and effective vaccines in late 2020…" was designed to mislead the public from the unassailable truth that no COVID-19 "vaccine" existed. Moreover, 21 CFR 312.7(a) prohibits the sponsor of the product or any person acting on behalf of the sponsor to promote an investigational new drug as either being safe or effective. Factually speaking, all COVID-19 drugs authorized only for emergency use were undergoing clinical trials and had not been licensed by the FDA by August 13, 2021.

449. Governor Brown willfully violated federal law because she knew that unless she forced individuals under her authority to participate in the use of one of the COVID-19

investigational drugs, they would not do so. She said, "According to polling from the Kaiser Family Foundation, some adults report they will only get vaccinated if required to do so for their employment." She knew that citizens were hesitant to participate in the use of investigational drugs and decided that she would re-write legislation passed by valid acts of Congress and enact her own mandates via fiat rule.

450. Governor Brown then unlawfully ordered employees of the State of Oregon to inject investigational COVID-19 drugs into their bodies no later than October 18, 2021. Not swayed by her oath of office and duties to protect the Constitutional rights of State employees under her authority, Governor Brown mandated that those individuals must also present to her their private health information, which she had no right to access. It is a purely private matter for any person considering participating in an investigational medical procedure. No governor is authorized by law to compel individuals under their authority to disclose whether or not they received an investigational drug such as Pfizer-BioNTech COVID-19 product. Additionally, the Governor issued a directive that the individual must provide "proof of vaccination" to continue employment. No individual had the legal or physical capacity to prove they were vaccinated because no such vaccine existed by which it could be proved.

451. Governor Brown then issued additional unlawful directives stating, "The Executive Branch is expected to make reasonable accommodations in order to comply with the Americans with Disabilities Act and Title VII of the Civil Rights Act, and state law equivalents, for individuals unable to be vaccinated due to disability, qualifying medical condition, or a sincerely held religious belief." This directive is an act of fraud because Governor Brown presented her Executive Order as having the authority to mandate investigational drug use and that the only means of exemption

were laws only pertaining to licensed products. Persons under her authority acted on that misrepresentation of authority leading to severe financial injury to Plaintiffs.

452. The requirement to obtain an exemption from medical investigation is an unlawful misrepresentation of authority by the Governor. Congress was explicit in that only the HHS Secretary has the authority to establish the conditions under which 21 U.S.C. §360bbb-3 products are administered. Not even the Secretary can require any person to participate in any activity under the section of law. Governor Brown intentionally ignored her constitutional restrictions, legal obligations, and her oath of office to protect the rights, privileges, and immunities of Plaintiffs. With utter disregard for the rule of law, Governor Brown assumed a power not granted to her for the intentional purpose of causing the Plaintiffs to surrender their constitutional rights resulting from their fears of economic loss.

453. Moreover, the Supremacy Clause, PREP Act, and Field Preemption doctrine for 21 U.S.C. §360bbb-3 removed her authority to enact any law, rule, ordinance, regulation, or custom that interfered with the emergency medical countermeasure program under the PREP Act, 21 U.S.C. §360bbb-3, and the CDC COVID-19 Vaccination Program Providers Agreement. Such disregard for the authority of Congress, the rights of Oregon citizens, and her oath of office shocks the conscience and demonstrates moral turpitude.

454. Governor Brown cannot produce a constitution, statute, regulation, treaty, or contract demonstrating her authority to establish conditions contrary to the Plaintiffs' right to refuse 21 U.S.C. §360bbb-3 medical countermeasures without consequence.

455. Now that Governor Brown established the false belief that the state and its agencies had the authority to place individuals under threat of penalty should they exercise their 21 U.S.C.

§360bbb-3 rights to refuse the investigational drugs, she informed agencies under her authority that they may "exceed the requirements of [her] Executive Order."

456. Patrick Allen and OHA wasted no time and issued their own COVID-19 mandate on August 5, 2021, and amended that rule on August 25, 2021. The primary difference between the two rules is that on August 5th, they allowed persons to test for COVID-19 virus using experimental diagnostics if they did not receive a COVID-19 investigational drug as a condition to continue employment. On August 25, 2021 Patrick Allen removed the right to test and unlawfully violated his oath of office and mandated that all healthcare workers and facilities must inject an investigational drug into their bodies or lose access to living wages, among other benefits.

457. The OHA provided the United States Government with an assurance via the Office of Human Research's Federal Wide Assurance agreement that it, nor any person in the State, would place individuals under "sanctions," "coercion," "undue influence," or "unjustifiable pressures" to participate in an investigational medical product, process, or study.[81]

458. The OHA has authority over medical facilities and health care workers to ensure its FWA agreement with HHS is fulfilled at all times.

459. Numerous State entities also gave their Federal Wide Assurances, such as:

(1) the Oregon Institute of Technology (FWA00009794),

(2) the University of Oregon (FWA00005914),

(3) the Oregon Health Authority (FWA00000520),

(4) Oregon State University (FWA00003920),

(5) Central Oregon Community College (FWA00026087), and

(6) Oregon Department of Corrections (FWA00027303), among others.

[81] The OHRP requires persons to abide by the ethical principles of the Belmont Report and 45 CFR 46 as a condition to receive federal funds. This agreement is known as the Federal Wide Assurance.

460. Additionally, nearly all medical facilities within the State also provided their assurances to HHS and received an agreement number denoting a legally binding agreement having third-party beneficiary rights. The Federal Wide Assurance exists exclusively to benefit third parties, and the third-party benefits contained within the Belmont Report and 45 CFR 46 must be met as a condition for promisors to receive federal funding.

461. Therefore, when the OHA issued a mandate on August 25, 2021 that healthcare facilities and workers must participate in a COVID-19 investigational drug under threat of penalty, Defendants knew that such a requirement was illegal, and it is expressly why the board refused, at all times pertinent, to discuss Plaintiffs' rights under 21 U.S.C. §360bbb-3.

462. To circumvent the Constitutional requirement to protect the statutory rights of Plaintiffs, the OHA echoed the voices of the past, arguing that the good of the mandate outweighed the rights of the victims. On August 25, 2021 the OHA stated,

(1) "In August of 2021, the B.1.617.2 (Delta) variant accounted for more than 98% of the COVID-19 infections in Oregon,"

(2) "The Delta variant is approximately 2-3-fold more transmissible than early wild-type COVID-19 variants,"

(3) "People infected with the Delta variant have higher viral loads and a shorter incubation period,"

(4) "There is emerging evidence that people infected with the Delta variant have similar viral loads regardless of vaccination status suggesting that even vaccine breakthrough cases may transmit this variant," and

(5) The Delta variant is causing a surge in unvaccinated cases and vaccine breakthrough cases."

463. OHA then stated that "being vaccinated, is therefore critical to prevent spread of Delta." OHA concealed the unassailable fact that no manufacturer of a COVID-19 EUA drug promoted their drugs as having an indication to prevent any variant of the SARS-CoV-2 (COVID-19) virus, nor did they state what level of protection their drugs would provide for each variant. OHA promoted the EUA COVID-19 drugs as having a legal indication to prevent the SARS-CoV-2 virus, and that they were safe despite the fact that the drugs were still undergoing clinical trials. In plain language, Governor Brown, Patrick Allen, and OHA created fraudulent information about the drug's legal indication that the FDA did not authorize.

464. However, on the same day that OHA issued a temporary administrative rule subjecting the State's entire medical profession to investigational drug use under threat of penalty, it published an alternative treatment to the Delta variant problem. OHA published "Monoclonal antibody treatment for COVID-19[82]," stating in part, (1) "Monoclonal antibody (mAb) treatments have the potential to save lives and relieve the burden of Oregon's health care system and are now available in subcutaneous (an injection) treatment, in addition to the intravenous (IV) formulation," (2) "the treatment is effective against the Delta variant, which is rapidly circulating in Oregon, and will help alleviate the intensifying strain on our hospital systems."

465. Therefore, 21 U.S.C. §360bbb-3 required OHA to ensure that healthcare facilities and workers were to be made aware of "alternatives" to the product being offered and the risks and benefits associated with the product. OHA had a viable alternative to treating the Delta variant,

[82] Monoclonal antibody treatment for COVID-19 (update) : Oregon Health News Blog. Oregon Health News Blog. Published August 25, 2021. Accessed August 19, 2023. https://covidblog.oregon.gov/monoclonal-antibody-treatment-for-covid-19-update/

which was producing miraculous results with little to no serious adverse events compared to the investigational products under its mandate.

466. OHA had one goal in mind when issuing their rule, and that goal was to comply with their boss's (Governor Brown's) political goal of subjecting the entire state, including Plaintiffs, to investigational COVID-19 drug use outside of their free will and voluntary consent and such misleading content was required to help them achieve their aim. This fact is evident in the concealment of 21 U.S.C. §360bbb-3's requirement to inform persons of the alternatives to the Pfizer-BioNTech COVID-19 EUA product, such as the monoclonal antibody treatments.

467. OHA stated, "The Authority finds that failure to act promptly will result in serious prejudice to the public interest…". One can only assume that OHA used the word "prejudice" under the legal meaning of "harm or injury that results or may result from some action or judgment" (Oxford Language). Factually speaking, by OHA enacting their rule, harm is exactly what occurred by their action. They harmed the Plaintiffs' Constitutional and statutory rights.

468. Defendants, acting under the color of law when executing their various unlawful COVID-19 vaccination requirements, deprived Plaintiffs of their Equal Protection and Due Process rights under the 14th Amendment, their statutory rights under 21 U.S.C. §360bbb-3, the Belmont Report, 45 CFR 46, and their third party beneficiary rights under the CDC COVID-19 Vaccination Program and the Federal Wide Assurance agreement under the State of Oregon.

469. The harms they suffered were extreme emotional distress, loss of access to living wages, retirement benefits, health care insurance, humiliation, loss of privacy, loss of dignity, feelings of equality, and equity within the State, among others.

470. However, OHA issued a penalty so severe that it surely invoked the Eight Amendment against "excessive fines." OHA warned the State's medical community that,

"Employers of healthcare providers or healthcare staff, contractors and responsible parties who violate any provision of this rule are subject to civil penalties of $500 per day per violation."

471. The U.S. Supreme Court referenced such debased government action in *Rex Trailer Co., Inc. v. United States*, 350 U.S. 148 (1956), stating that government action "transformed what was clearly intended as a civil remedy into a criminal penalty."

472. In *Hudson v. United States*, 522 U.S. 93, 118 S.Ct 488, 139 L.Ed.2d 450 (1997), the court held:

> The Due Process and Equal Protection Clauses already protect individuals from sanctions which are downright irrational. *Williamson v. Lee Optical of Okla., Inc.,* 348 U.S. 483, 75 S.Ct. 461, 99 L.Ed. 563 (1955). The Eighth Amendment protects against excessive civil fines, including forfeitures. *Alexander v. United States,* 509 U.S. 544, 113 S.Ct. 2766, 125 L.Ed.2d 441 (1993); *Austin v. United States,* 509 U.S. 602, 113 S.Ct. 2801, 125 L.Ed.2d 488 (1993)."

473. Fining a nurse "$500 per day per violation" for exercising a federal statutory right is not only a direct assault on the Eighth Amendment, but it is also an unlawful act against the authority of the United States Congress. The purpose of the excessive and unlawful fine was to coerce individuals into accepting the status quo without questioning the legality or authority behind the mandates.

474. A "sanction" exists to coerce obedience to the law, not as a tool to amend laws with which fiat rulers unlawfully disagree. Defendants sanctioned Plaintiffs for obeying the laws of the United States Congress because those laws interfered with the goals, political aspirations, and personal desires of Defendants.

475. The actions by Defendants represent a clear and present danger to the Constitutional form of government within the State of Oregon. The complete wanton disregard for the health,

safety, and rights of citizens under their authority truly shocks the conscience and is considered outrageous in light of a modern society operating under a Republic form of government.

476. Defendants' decision to usurp the authority of Congress and legislate by fiat has created a Constitutional crisis within the State of Oregon by establishing two classes of citizens. One class is made up of those who are under the protection of the Constitution and enjoy the liberties it secures for them. The other class comprises those who exercised a statutory right, but a right Defendants disagreed with, resulting in a form of government where Plaintiffs are deprived of liberties and property without due process and are deprived of the equal protection of laws.

477. When the Plaintiffs refused the fiat dictates of Defendants, Plaintiffs were demoted to that of a second-class citizenry and stripped of their Constitutional protections, privileges, and immunities. Defendants demonstrated a willful and wanton disregard for known laws protecting Plaintiffs from medical research abuses, which is an outrage in light of our nation's abuse of human rights discussed above.

478. Defendants, at all times pertinent, intentionally refused to allow citizens under their authority to give their legally effective informed consent as if Congress never mandated that legal exercise before 21 U.S.C. §360bbb-3 product administration.

479. PeaceHealth's COVID-19 Vaccine Requirement Policies, issued in August of 2021 states, "PeaceHealth recognizes that immunization with a safe and effective vaccine is a proven strategy to reduce COVID-19 related illness for its HCWs, patients, and communities. Only HCWs with a qualified and accepted medical or religious exemption may decline COVID-19 vaccination."[83]

[83] See Exhibit B, PeaceHealth's COVID-19 Vaccine Requirement Policy.

480. The statement that "immunization with a safe and effective vaccine is a proven strategy to reduce COVID-19 related illness" was an intentionally misleading statement and a violation of federal law (21 CFR 312.7(a)).

481. There does not and did not exist a COVID-19 "vaccine" available to Plaintiffs. No COVID-19 drug manufacturer has ever claimed that their investigational drugs would immunize Plaintiffs from the SARS-CoV-2 (COVID-19) infection. The statement in the preceding paragraph does not claim that there actually existed a "safe and effective vaccine." Still, it misleadingly implies that the statement applies to the drugs under their mandate, making an average person conclude that the COVID-19 EUA drugs were "safe and effective vaccines." The statement demonstrates a wanton disregard for Plaintiffs' rights, autonomous beliefs, and health goals when involving them in the use of an investigational drug.

482. The policy statement, "Only HCWs with a qualified and accepted medical or religious exemption may decline COVID-19 vaccination," severely violates federal law and the Plaintiffs' constitutional and statutory rights.

483. Federal law explicitly states that the individual possesses the power to accept or refuse EUA countermeasure participation. The Plaintiffs' statutory authority to either accept or refuse is absolute and not conditioned upon them seeking an exemption. One must question if Defendants believe they also have the authority to impose exemption conditions on the right to accept.

484. PeaceHealth is legally sophisticated with laws and regulations involving investigational drugs and knew in advance that the requirement to seek an exemption violated those laws.

485. PeaceHealth and its policymakers knew they were bound by treaty, law, regulation, and contract to create a legally approved environment ensuring no person was sanctioned for not participating in available COVID-19 investigational drugs.[84],[85]

486. PeaceHealth was also well aware that administering an investigational medical product to as little as only one individual requires IRB oversight and compliance with 45 CFR 46, the FWA, and other laws protecting humans involved in medical research activities.[86]

487. PeaceHealth's immunization policy was built upon intentionally fraudulent misrepresentation of facts and illegal statements.

488. PeaceHealth's policy stated, "Except as set forth below, HCWs are considered fully vaccinated two weeks after they have received Pfizer or Moderna second dose, Johnson & Johnson single dose or other U.S. Food and Drug Administration (FDA) Emergency Use Authorization (EUA), FDA approved, FDA authorized or World Health Organization (WHO) emergency use/approved COVID-19 vaccine/series."

489. PeaceHealth lacked legal authority to convey that a person would be "fully" protected when and if they were administered investigational drugs undergoing clinical trials and having no legal indication to treat, cure, or prevent any known disease. The statement was dangerous because the medical community had not even come close to establishing how many contraindications the EUA drugs had with existing medical conditions or other drugs.

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[84] "When informed consent is required, it must be sought prospectively, and documented to the extent required under HHS regulations at 45 CFR 46.117." - Informed Consent FAQs. HHS.gov. Published 2018. Accessed June 8, 2023. https://www.hhs.gov/ohrp/regulations-and-policy/guidance/faq/informed-consent/index.html

[85] "Because individuals receiving urgent or emergent medical care frequently may be vulnerable to coercion or undue influence, even if temporarily, additional protections may be required to ensure the subject's consent to participate in research is truly voluntary and sought under circumstances that minimize the possibility of coercion or undue influence (45 CFR 46.111(b), (45 CFR 46.116)." – *Id.*

[86] "This policy applies to all research involving human subjects conducted, supported, or otherwise subject to regulation by any Federal department or agency that takes appropriate administrative action to make the policy applicable to such research. This includes research conducted by Federal civilian employees or military personnel" - 45 CFR 46.101(a) (Basic HHS Policy for Protection of Human Research Subjects)

490. PeaceHealth disseminated misinformation when conveying that if Plaintiffs received the recommended doses of one of the unlicensed, investigational medical products, they would become "fully vaccinated" against the virus. No COVID-19 drug manufacturer claims to immunize any person from any COVID-19 variant.

491. CEO Liz Dunne's COVID-19 policy further stated, "PeaceHealth recognizes that immunization with a safe and effective vaccine is a proven strategy to reduce COVID-19 related illness for its HCWs." This statement violates 21 CFR 312.7(a) by implying that the investigational new drugs are safe and effective for the purpose under which they are being investigated.

492. CEO Dunne attempted to shield herself and PeaceHealth from future litigation by creating a document titled "Covid-19 Vaccine Acknowledgment (caregivers)." It includes the following statement to which Plaintiffs were to attest by signing the document: "I am making the choice to get the COVID-19 vaccine on my own and freely. I know I have the option to refuse the vaccine or talk to my physician prior to receiving the vaccine. I ask that the vaccine be given to me."

493. CEO Dunne holds the proverbial gun to the heads of Plaintiffs, telling them that she will destroy their financial lives if they do not participate in one of the COVID-19 investigational drugs, and then compels them under duress to sign a document before receiving the product, or the product's Fact Sheet, attesting that such action was under their free will and voluntary consent. CEO Dunne's actions are expressly why Congress passed the 1973 National Research Act.

494. PeaceHealth further shocks the conscience when mandating, "Fully vaccinated HCWs will be provided a badge sticker by PeaceHealth that indicates vaccination…The badge sticker must be visibly attached to their PeaceHealth name badge or such other name badge used

to be on-site at a PeaceHealth location and not cover the HCW's name or other pertinent identifying information."

495. By mandating that the "vaccinated" wear a badge sticker, PeaceHealth not only identified the "vaccinated," but they identified by omission, and thus publicly shamed, the "unvaccinated."

496. Moreover, PeaceHealth is a "covered" entity under HIPAA. It is well aware that it is a criminal violation of law to disclose Private Health Information publicly. The requirement for "vaccinated" employees to wear a badge sticker violated the HIPAA rights, not only the "vaccinated," but the "unvaccinated," including Plaintiffs herein.

497. Moreover, the HHS developed standards for the Privacy of individually identifiable health information. It issued a final rule in 2002[87] stating in part: "a covered entity must remain cognizant of its dual roles as an employer and as a health care provider, health plan, or health care clearinghouse. Individually identifiable health information created, received, or maintained by a covered entity in its health care capacity is protected health information. It does not matter if the individual is a member of the covered entity's workforce or not. Thus, the medical record of a hospital employee who is receiving treatment at the hospital is protected health information and is covered by the Rule, just as the medical record of any other patient of that hospital is protected health information and covered by the Rule. The hospital may use that information only as permitted by the Privacy Rule, and in most cases will need the employee's authorization to access or use the medical information for employment purposes." (Emphasis added)

[87] 67 Fed. Reg. 53191

498. PeaceHealth willfully engaged in criminal violations of HIPAA laws[88] for the express purpose of subjecting Plaintiffs to public pressures in hopes of causing them to surrender their constitutional protections and receive a COVID-19 investigational drug so that PeaceHealth can reap the financial benefits of the State's immunization program and for what PeaceHealth believed to be commercially advantageous to PeaceHealth.

499. As a direct result of PeaceHealth's badge sticker mandate, when Plaintiffs thereafter arrived for their work shifts, PeaceHealth personnel ridiculed Plaintiffs for not having a badge sticker.

500. Even more shocking to the conscience, PeaceHealth patients were all made aware of the badge sticker's meaning, causing patients to require that Plaintiffs leave the room, making Plaintiffs feel as if they were a stigma to society, all because Plaintiffs chose to exercise their constitutionally protected rights to refuse administration of a drug that potentially had significant health and legal consequences for their lives.

501. PeaceHealth was disgusted that Plaintiffs chose the option to refuse, and engaged in a "scorched earth" policy to make Plaintiffs pay for that choice no matter the laws PeaceHealth had to violate to achieve their unlawful goal.

502. PeaceHealth chose to ignore well-known and practiced laws and regulations to subject Plaintiffs to outrageous conduct to force them to surrender their constitutional protections of being treated equally before the law and being afforded due process.

503. Shockingly, PeaceHealth provided assurance to its employees that "Proof of vaccination shall be maintained by PeaceHealth in a confidential file for the time prescribed by

[88] 42 U.S.C. § 1320d-6(b)(3) "if the offense is committed with intent to sell, transfer, or use individually identifiable health information for commercial advantage, personal gain, or malicious harm, be fined not more than $250,000, imprisoned not more than 10 years, or both."

law and will be made available to regulatory bodies upon request to the extent required or allowed by law. PeaceHealth may rely on third parties (e.g., contractors) to assist in maintaining proof of vaccination. In such case, such third party shall sign a declaration outlining its responsibility to help enforce this policy and applicable state law and provide PeaceHealth a copy of said proof of vaccination upon request as allowed by law."

504. Defendants' actions subjected Plaintiffs to that of a second-class citizen, causing them to fear for their physical safety, and financial security, and loss of believing their rights were as important and equal to other employees.

505. PeaceHealth's COVID-19 policy states, "HCWs will be subject to COVID-19 vaccination recommendations published by the CDC, including but not limited to a booster dose in the future. HCWs will comply with the requirements issued by PeaceHealth as it relates to any new COVID-19 vaccination recommendations, subject to the HCWs' right to file a qualifying medical or religious exemption."

506. PeaceHealth willfully ignored its contractual obligations under the CDC COVID-19 Vaccination Program Provider agreement, its IRB legal obligations, its FWA agreement, and 21 U.S.C §360bbb-3 duties by mandating that which the federal government only recommended. The CDC provided many recommendations that may not come under any mandate because such requirements would violate well-established laws.

507. When PeaceHealth declared that it would subject employees "to a booster dose in the future," it demonstrated an intentional disregard for the law because it chose to amend laws enacted by a valid act of Congress governing the administration of investigational drugs. No Constitution, statute, regulation, contract, or ordinance affords PeaceHealth authority to subject

any person to an investigational drug at any time. As demonstrated in this Complaint, such actions are not only restricted, they are unlawful.

508. PeaceHealth continued by stating, "HCWs that have received the vaccine but have not achieved a fully vaccinated status; are fully vaccinated but refuse to wear a vaccine badge sticker; or who are granted an exemption and a reasonable accommodation; or who are granted an exception as allowed below will be subject to the Universal Masking Policy, Physical Distancing Policy and other PeaceHealth PPE, infection control, and safety policies applicable to COVID-19, which include the requirement of routine PCR testing for COVID-19 infection." (Emphasis added).

509. Even employees who were "fully vaccinated but refuse[d] to wear a vaccine badge sticker" would be denied their Constitutional rights of equal protection and privacy rights under Oregon's common law. They, too, would be subjected to "the Universal Masking Policy, Physical Distancing Policy and other PeaceHealth PPE, infection control, and safety policies applicable to COVID-19, which include the requirement of routine PCR testing for COVID-19 infection." Such tyrannical disregard for the rule of law is outrageous in modern America.

510. PeaceHealth violated the Fourteenth Amendment rights of Plaintiffs when stating they would be subjected to a "Universal Masking Policy, Physical Distancing Policy and other PeaceHealth PPE, infection control, and safety policies applicable to COVID-19, which include the requirement of routine PCR testing for COVID-19 infection" should they choose one of the two federally protected options under 21 U.S.C. 360bbb-3 because the option to refuse is as equal as the option to accept. Moreover, the COVID-19 testing articles were also under EUA status, prohibiting PeaceHealth from including them under a mandate.

511. PeaceHealth placed Plaintiffs under undue influence, illegal sanctions, and moral duress when stating, "Employed HCWs who fail to comply with the requirements of this policy will be placed on unpaid leave until compliant. Failure to comply may result in corrective action up to and including termination of employment."

512. The moral duress is evident in the fact that Plaintiffs built careers, lives, and dreams around working at PeaceHealth. Defendants knew the reliance the Plaintiffs had on PeaceHealth for employment. With callous disregard for the general welfare of Plaintiffs' lives, Defendants demoted Plaintiffs to second-class citizen status. They deprived them of their rights to continue earning a living at PeaceHealth, all because Defendants disagreed with Plaintiffs' chosen option under 21 U.S.C. 360bbb-3. Defendants disregarded the rule of law as established by Congress, and under fiat authority, wrote their COVID-19 vaccination policy with the backing of Oregon's Governor as cover, knowing that Attorney General Bob Ferguson would not enforce the laws on the books.

513. PeaceHealth engaged in additional discriminatory acts when declaring, "Non-employed HCWs who fail to comply with the requirements of this policy will be excluded from engaging in work for Health Care Settings operated, owned, leased, or controlled by PeaceHealth." The statement violates the legal conditions PeaceHealth voluntarily agreed to under 21 U.S.C. 360bbb-3, FWA, IRB, 45 CFR 46, and the CDC COVID-19 Vaccination Program Providers Agreement.

514. 21 U.S.C. §360bbb-3 required Defendants to provide Plaintiffs with information about a medical countermeasure regarding the risks, benefits, alternatives, and risks and benefits to those alternatives when they were offered participation in a medical countermeasure authorized

by the Secretary. Defendants refused to inform Plaintiffs of this information in any company policy requiring EUA drug participation.

515. The law also requires Defendants to inform Plaintiffs of their right to refuse, which is a legal right. Defendants intentionally withheld this information to increase acceptance of an EUA drug, to which 15,000 or more other employees agreed.

516. Moreover, when an individual agrees to participate in a PREP Act activity or to use a 21 U.S.C. §360bbb-3 medical countermeasure, they agree to

A. forfeit litigation rights in courts when sustaining an injury,

B. assume greater risks to their health, safety, and legal rights,

C. allow their private identifiable information to be known by unknown persons and for unknown reasons,

D. allow the data collected about their involvement with the product or activity to be utilized by researchers for unknown purposes and for eternity.

517. Defendants intentionally refrained from informing employees of the consequence of accepting an investigational drug only authorized for emergency use under PREP Act authority.

518. Refusing to inform Plaintiffs of this information was a violation of 21 U.S.C. §360bbb-3 and done to induce participation outside of Plaintiffs' free will and voluntary consent.

519. Defendants knew that their actions violated every known law relating to private health information, investigational drug administration, emergency use of investigational drugs, their duties under the CDC COVID-19 Vaccination Program Provider Agreement, FWA, and their personal Institutional Review Boards.

520. Instead of continuing in the lawful application of their duties which they practiced for decades, they decided to implement a personal agenda of cashing in on the government's COVID-19 gold mine at the expense of Plaintiffs' rights as protected by the Constitution.

521. Moreover, Defendants chose to ignore the Constitution and federal law by denying the authority of Congress. They chose to establish their own mandates, conditions, and laws in defiance of contractual obligations made to the CDC, HHS, and those under the authority of those entities.

522. The moral turpitude of PeaceHealth, Liz Dunn, Todd Salnas, and Doug Koekkoek, is aptly demonstrated by the unlawful and debased actions taken against Plaintiff Alisha Carter. Mrs. Carter exercised her right via Governor Brown, OHA, and PeaceHealth to seek a medical exemption. Carter presented a notice from her doctor on August 6th, 2021, stating, "Alisha Carter is under my care and is currently in her first trimester of pregnancy. I am recommending that she wait on getting the COVID vaccination until she is in the 2nd trimester, which will be after the August 31st cut off date. Please allow her through September to be vaccinated."

523. PeaceHealth replied to Carter on August 27th, 2021, stating in part, (1) "Your COVID-19 vaccination exemption request has been denied," (2) "Possible reasons for denial include: Documentation does not support the claim of a medical contraindication exemption."

524. At all times pertinent, Defendants never issued policy requirements of Plaintiffs obtaining a medical exemption on proving a medical contraindication with one of the mRNA drugs. Since there are over six trillion possible contraindications, such proof (see, *supra*) would be impossible to provide. Most shocking is that a medical professional provided written notice to PeaceHealth human resources that an employee should not participate in a medical procedure, and human resources ignored that doctor's advice which was based on the doctor's personal and first-hand knowledge of Mrs. Carter's medical condition. The reason why Mrs. Carter should not participate is a very personal reason, and PeaceHealth had no authority to intrude into Carter's personal affairs by requiring additional documentation.

525. More shocking is that OHA provided Chief Executive Todd Salnas the authority to wait until October 18th, 2021, before meeting their COVID-19 mandate. Moreover, PeaceHealth informed Todd Salnas and his human resources that they had until October 15th to comply with the mandate. In other words, there was no rational basis for not providing Mrs. Carter with a medical exemption. The reason was irrational because it was designed to convey the message to Plaintiffs that under no circumstance would Todd Salnas allow them an exemption from Defendant's unlawful mandate even if it brought harm to an unborn child and a pregnant mother.

526. PeaceHealth has routinely administered investigational drugs under 21 U.S.C. §360bbb *et seq* protocols for decades and knows that a pregnant woman cannot be under threat of penalty to participate in an investigational drug program. Moreover, if the drug involves more than minimal risk to the mother or the fetus,[89] then the mother is prohibited from participating.

527. 21 CFR 50.3(k) (Protection of Human Subjects; Definitions) defines minimal risk as "the probability and magnitude of harm or discomfort anticipated in the research are not greater in and of themselves than those ordinarily encountered in daily life or during the performance of routine physical or psychological examinations or tests."

528. At all times pertinent, PeaceHealth knew that myocarditis and pericarditis were risks of taking the Pfizer BioNTech COVID-19 EUA investigational drug.[90]

529. PeaceHealth was aware that Pfizer itself published data stating that its COVID-19 EUA drug caused adverse events including but not limited to syncope, diminished immune response, lymphadenopathy, anaphylaxis, pruritus, urticaria, angioedema, vomiting, dizziness, all of which could pose serious medical trauma to the mother and death to the embryo or fetus.

[89] 45 CFR 46.204(b)
[90] See Exhibit C, Fact Sheet for Recipients and Caregivers

530. PeaceHealth's callous disregard for the safety and well-being of expectant mothers is unheard of by healthcare professionals. Its arrogant position that pregnant mothers who are also employees may not be granted a medical exemption from investigational drug use is unlawful.

531. Moreover, PeaceHealth knew that no expectant mothers could provide "medical documentation" to support an exemption solely based on pregnancy because no COVID-19 drug manufacturer conducted, much less concluded, studies of the product's safety for pregnant women, with Pfizer's non-interventional study on the effects of its BioNTech COVID-19 Vaccine Exposure during pregnancy not being complete until June 30, 2025.[91]

532. PeaceHealth used its positions of power to compel the participation of pregnant women in the use of potentially dangerous investigational drugs and medical products with willful, wanton, and shocking disregard for their safety, health, and rights.

533. PeaceHealth, its policymaker, Liz Dunne, and Todd Salnas' moral turpitude [92]are evidenced by willful acts of disregarding federal and state laws, fraudulent misrepresentations of facts, publishing lies, and intentionally misleading employees about the legal distinctions between a licensed vaccine and a drug undergoing clinical investigation having no legal indication to treat, cure, or prevent any known disease.

534. At no time before, during, or after PeaceHealth's immunization policy was issued did any Defendant attempt to educate Plaintiffs of their rights to refuse the administration of an investigational new drug without incurring a penalty or losing a benefit to which they were otherwise entitled. The lack of such communication is an act of intentional fraud.

[91] Gmbh B. Our STN: BL 125742/0 BLA APPROVAL.; 2021. https://www.fda.gov/media/151710/download

[92] "This phrase is used to describe the violation of decent, moral and honest behavior and an act of depravity or vileness." Black's Law Dictionary 2nd Ed.

535. The continual unlawful promotion that the products were safe and effective and that it was the "duty" of healthcare workers to be injected with those products implied that the Plaintiffs did not care about the welfare of patients or their community. PeaceHealth affirmed this position when it informed Oregon State Employment Department that Plaintiffs engaged in misconduct (i.e., exercising a federal right to refuse the EUA product) and, thus, must not be allowed to receive unemployment benefits.

536. Mrs. Carter applied for unemployment benefits and was denied solely on the basis of her exercising her 21 U.S.C. §360bbb-3 rights to refuse. The Oregon Employment Department informed Mrs. Carter on October 19, 2021 that, "You claimed benefits for the period September 19, 2021 through October 16, 2021. To be eligible for benefits you must available to accept work as a physical therapist in Eugene and Springfield, OR. The customary hours you must be available are Sunday through Saturday 8am-5pm. You are not available for work as a physical therapist within your labor market, nor are you seeking work within your labor market."

537. Mrs. Carter was denied employment, though she desperately sought it out, because of her vaccination status of not injecting an investigational COVID-19 drug into her body. The State enacted a policy denying Mrs. Carter the right to work and then enforced that policy by denying her access to public benefits. Such actions are a clear violation of the Unconstitutional Conditions Doctrine.

538. PeaceHealth forced Plaintiffs to utilize their paid time off after placing them on unpaid administrative leave. PeaceHealth did not force other employees to use their paid time off after accepting the injection of an investigational COVID-19 drug. Such discrimination is a blatant violation of Plaintiffs' Equal Protection and Due Process rights under the Fourteenth Amendment.

539. Not only did PeaceHealth's actions result in lost economic benefits, but the actions resulted in defamation of character of Plaintiffs who worked for decades without a blemish on their employment records, the same Plaintiffs who continually were called upon to give up their weekends, holidays, and other pleasures of life to sweat, toll, and labor for the express purpose of providing life-saving medical care to the community. The assault on Plaintiffs' character was designed to place fear into other employees and in the hopes of causing those employees to surrender their rights conferred upon them by Congress.

540. It shocks the conscience to think that in modern-day America, Liz Dunne, Chief Executive Officer of one of Oregon's largest hospital networks, employing thousands of healthcare heroes, would lie to the State about the conduct of her employees all to avoid paying higher unemployment insurance when it was her actions acting under color of law that led to the State having a higher financial burden resulting from needless loss of wages by Plaintiffs.

541. CEO Liz Dunne willfully and intentionally deprived employees of their rights and then lied to the State about her misconduct in an effort to soften the financial blow to PeaceHealth's bottom line. Such actions are outrageous.

542. The abusive action shocks the conscience when one realizes that Plaintiffs have spent hundreds of combined years laboring in love to wipe the tears of pain from injured patients, reassuring little ones, and comforting those who lost the love of their lives only moments prior.

543. Plaintiffs were targeted for abuse by PeaceHealth to display them as an example for the express purpose of placing fear into the hearts of other PeaceHealth employees resulting in them surrendering their Constitutional protections and statutory rights.

544. The actions of Defendants were atrocious, intolerable, and so extreme and outrageous as to exceed the bounds of decency.

## XXIII. Recap of the Essentials

545. Due to the volume of information in this Complaint resulting from its novelty and substantial impact on the nation, it is prudent to recapitulate the essential points of the Complaint into one section.

546. To protect the safety and health of the public, Congress mandates that "no person shall introduce or deliver for introduction into interstate commerce any new drug unless an approval of an application" by the FDA is issued. (21 U.S.C. §355(a))

547. However, Congress recognized the potential benefits of providing access to unlicensed drugs, biologics, and devices to persons under emergency conditions and enacted legislation to provide "expanded access to unapproved therapies and diagnostics."[93] (21 U.S.C. § 360bbb *et seq.*)

548. In 2005, Congress passed Project Bioshield to provide emergency broad access (large population) to unlicensed uses of drugs, biologics, and devices during a declared emergency. Congress declared

A. that only the Secretary "may authorize the introduction into interstate commerce, during the effective period of a declaration…, a drug, device, or biological product intended for use in an actual or potential emergency.

B. that the Secretary "shall, for a person who carries out any activity for which the authorization is issued, establish such conditions on an authorization under this section as the Secretary finds necessary or appropriate to protect the public health…"

[93] "The Secretary may, under appropriate conditions determined by the Secretary, authorize the shipment of investigational drugs or investigational devices for the diagnosis, monitoring, or treatment of a serious disease or condition in emergency situations." 21 U.S.C. § 360bbb(a)

C. the Secretary has no "authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section…"

549. In 2005 Congress passed the PREP Act[94] which provided the following regarding preemption of state law:

> (8) During the effective period of a declaration under subsection (b)…no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that—
>
> (A) is different from, or is in conflict with, any requirement applicable under this section; and
>
> (B) relates to the…administration…of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.].

550. Therefore, via the PREP Act and 21 U.S.C. §360bbb-3, Congress expressly prohibits Defendants from:

A. interfering with the authority of the Secretary,

B. establishing a condition not authorized by the Secretary,

C. establish conditions contrary to the Secretary and the congressional statute under 21 U.S.C. §360bbb-3,

D. mandate participation in any 21 U.S.C. §360bbb-3 product or PREP Act activity,

E. interfere with an individual considering participation in a 21 U.S.C. §360bbb-3 product or PREP Act activity,

F. penalize a person refusing to participate in a PREP Act product or activity or 21 U.S.C. §360bbb-3 product.

551. The executive branch of the United States Government purchased all COVID-19 licensed and EUA drugs using federal funds. Congress expressly prohibits the federal government

[94] 42 USC 247d-6d and 42 USC 247d-6e

and any person acting on behalf of the federal government from applying outside pressures on individuals to participate in 21 U.S.C. §360bbb-3 products.

552. Therefore, Defendants not only lacked statutory authority to alter the employment of Plaintiffs choosing one of the two federally funded COVID-19 options to accept or refuse, but the government prohibited them from interfering with that choice. That prohibition is under the PREP Act, 21 U.S.C. §360bbb-3, and the CDC COVID-19 Vaccination Program Provider Agreement.

553. Congress expressly provided the individual with two equal choices; accept or refuse.

554. The 14th Amendment guarantees that all persons shall enjoy the equal protection of the laws. Defendants violated the equal protection of Plaintiffs when only penalizing persons exercising the option to refuse. The 14th Amendment guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property. The defendants' unwillingness to respect the authority of Congress by recognizing the statutory right of Plaintiffs to refuse 21 U.S.C. §360bbb-3 products and PREP Act activities without consequence nullifies the substantive and procedural due process rights of Plaintiffs.

555. Defendants committed arrestable offenses under 18 U.S.C. §§ 241, 245 by interfering with Plaintiffs' Constitutional and statutory rights and Plaintiffs' involvement with a federally funded program.

556. Defendants' actions demonstrate a willful and wanton disregard for the rights of the Plaintiffs, Congress, the Constitution, and their assurances under the CDC COVID-19 Vaccination Program Provider and FWA agreements not to subject Plaintiffs to medical experimentation outside of their free will and voluntary consent.

557. Defendants' actions include, but are not limited to:

A. intentionally and unlawfully misrepresented their authority to Plaintiffs to cause them to surrender their constitutional and statutory rights.

B. engaged in acts of coercion, undue influence, and retaliation, creating a hostile work environment.

C. placing Plaintiffs under moral duress[95] knowing they exclusively relied on Defendants for access to living wages.

D. segregating Plaintiffs under discriminatory acts upon Plaintiffs exercising their absolute right to refuse investigational new drugs.

E. unlawfully altering Plaintiffs' employment schedules under coercive acts to punish them for exercising their absolute right to refuse investigational new drugs.

F. unlawfully attempting to coerce Plaintiffs to engage in a legally binding agreement under the terms and conditions (21 U.S.C. §360bbb-3 and PREP Act) established by the United States Congress outside their free will and voluntary consent.

G. engaging in acts of moral turpitude against Plaintiffs' rights, safety, and health.

H. unlawfully terminating and or altering Plaintiffs' employment when Plaintiffs exercised a legal right or privilege.

I. willfully and with the intent to deprive, failed to pay wages to Plaintiffs since the date Defendants unlawfully placed Plaintiffs on administrative leave.

558. At all times pertinent, Plaintiffs did not knowingly submit to the deprivation of labor, wages, bonuses, healthcare, retirement funding, employment, or other benefits.

[95] Moral duress consists of imposition, oppression, undue influence, or the taking of undue advantage of the business or financial stress or extreme necessity or weakness of another. *Lafayette Dramatic Productions v. Ferentz*, 306 Mich. 193, 9 N.W.2d 57, 66; See also Black's Law Dictionary, Sixth Edition, p. 1008.

## XXIV. Legal Claims

559. The facts described above constitute violations of several of the rights guaranteed to Plaintiffs by the United States Constitution, federal statutes, and treaties. These violations are actionable under 42 U.S.C. § 1983 because the Defendants acted under color of state law when issuing their COVID-19 vaccination requirements and administrating the CDC COVID-19 Vaccination Program Provider Agreement.

## COUNT ONE

## Subjected to Investigational Drug Use - 42 U.S.C. § 1983

569. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

570. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR Part 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, 10 U.S.C. § 980, EUA Scope of Authorization letters, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

571. 45 CFR 46.116(b)(8) states: "A statement that participation is voluntary, refusal to participate will involve no penalty or loss of benefits to which the subject is otherwise entitled, and the subject may discontinue participation at any time without penalty or loss of benefits to which the subject is otherwise entitled."

572. The Belmont Report declares, "An agreement to participate in research constitutes a valid consent only if voluntarily given. This element of informed consent requires conditions free of coercion and undue influence."

573. 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) contains a required condition of the Secretary "to ensure that individuals to whom the product is administered are informed — "of the option to accept or refuse administration of the product."

574. Article VII of the ratified International Covenant on Civil and Political Rights (ICCPR) Treaty affirms that "…no one shall be subjected without his free consent to medical or scientific experimentation."

575. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, unlawfully subjected Plaintiffs to the use of investigational medical products under threat of penalty outside of their free will and voluntary consent as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

**COUNT TWO**

**Unconstitutional Conditions Doctrine - 42 U.S.C. § 1983**

576. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

577. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR §46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

578. The courts have long held that an individual "may not barter away his life or his freedom, or his substantial rights." Insurance Company v. Morse, 87 U.S. 445 (1874)

579. "It would be a palpable incongruity to strike down an act of state legislation which, by words of express divestment, seeks to strip the citizen of rights guaranteed by the federal Constitution, but to uphold an act by which the same result is accomplished under the guise of a surrender of a right in exchange for a valuable privilege which the state threatens otherwise to withhold. It is not necessary to challenge the proposition that, as a general rule, the state, having power to deny a privilege altogether, may grant it upon such conditions as it sees fit to impose. But the power of the state in that respect is not unlimited; and one of the limitations is that it may not impose conditions which require the relinquishment of constitutional rights. If the state may compel the surrender of one constitutional right as a condition of its favor, it may, in like manner, compel a surrender of all. It is inconceivable that guaranties embedded in the Constitution of the United States may thus be manipulated out of existence (emphasis added)". Frost Trucking Co. v. R.R. Com, 271 U.S. 583, 593-94 (1926)

580. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, manipulated the Constitutional rights of Plaintiffs out of existence as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

**COUNT THREE**

**Equal Protection - 42 U.S.C. § 1983**

581. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

582. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR §46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of

Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

583. The Fourteenth Amendment to the U.S. Constitution guarantees equal protection of the laws.

584. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their equal protection rights as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

**COUNT FOUR**

**Due Process - 42 U.S.C. § 1983**

585. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

586. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

587. The Due Process Clause of the Fourteenth Amendment to the U.S. Constitution guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property.

588. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the

Plaintiffs of their substantive and procedural due process rights as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

**COUNT FIVE**

**Deprivation of Rights Under Color of Law - 42 U.S.C. § 1983**

**Spending Clause Doctrine**

589. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

590. The CDC COVID Vaccination Program Provider Agreement, 45 CFR §46.122, 10 U.S.C. §980, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

591. 45 CFR §46.122 provides: "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied."

592. 10 U.S.C. §980 states: "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless – the subject's informed consent is obtained in advance…"

593. The United States Government fully funds the CDC COVID-19 Vaccination Program's research activities involving human subjects. Additionally, the United States Government fully funds the required research activities assigned by the HHS Secretary for Pfizer-BioNTech COVID-19 Vaccine, Moderna, Janssen, and Novavax. Those specific research protocols are outlined in the Secretary's Scope of Authorization contained in the Emergency Use Authorization for each COVID-19 drug.

594. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes and regulations, refused to obtain the legally effective informed consent of the Plaintiffs in violation of spending legislation as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

## COUNT SIX

## Breach of Contract, Third Party Beneficiary

595. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

596. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR 46, 21 U.S.C. §360bbb-3, Title 21 of the US Code, the EUA Scope of Authorization letter clearly and unambiguously create third-party beneficiary rights.

597. The Defendants' actions described above, individually and/or collectively, and in derogation of the CDC COVID-19 Vaccination Program Provider Agreement, violated the intended benefits conferred upon the Plaintiffs through the terms and conditions of the CDC COVID Vaccination Program Provider Agreement as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

## COUNT SEVEN

## Intentional Infliction of Emotional Distress

598. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

599. When the United States Congress refused to allow, Defendants, to apply consequences to Plaintiffs refusing to participate in the use of COVID-19 investigational drugs,

Defendants engaged in a scorched earth policy and inflicted with malicious intent severe emotional distress to the fullest extent that one in their positions of authority and power could inflict to the detriment of Plaintiffs' emotional well-being.

600. Intentional infliction of emotional distress is proven by (1) demonstrating that Defendants engaged in extreme and outrageous conduct, (2) Defendants' actions caused severe emotional distress, (3) Defendants intentionally or recklessly caused the emotional distress, (4) that Plaintiffs are the direct recipients of the conduct.

601. The facts and the Defendants' conduct committed with gross negligence, reckless, or intent, as described above in the complaint, give rise to a claim of Outrage under the common law of the State of Oregon against the Defendants for the damages described in Paragraphs 605 through 612, *infra*.

## COUNT EIGHT

## Implied Private Right of Action 21 U.S.C. §360bbb-3

602. Plaintiffs hereby incorporate by reference the allegations contained in Paragraphs 1 through 30 and 36 through 559, as if fully set forth herein.

603. Should the court not agree that PeaceHealth was engaged in State Action, Plaintiffs claim that 21 U.S.C. §360bbb-3 contains an implied private right of action pursuant to *Cannon v. University of Chicago*, 441 U.S. 677 (1979), *Wilder v. Virginia Hosp. Ass'n*, 496 U.S. 498 (1990), and *Cort v. Ash*, 422 U.S. 66 (1975).

604. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty have deprived the Plaintiffs of their explicit right to refuse the administration of an emergency use authorized drug

and/or medical product without penalty as described in the above facts, thereby causing them damages described in Paragraphs 605 through 612, *infra*.

## XXV. Damages Recoverable and Demanded

605. The following paragraphs are hereby incorporated by reference into Counts I through IX, as if set forth there *in extenso*.

606. As a direct and proximate result of the Defendants' unreasonable and unlawful actions, Plaintiffs have suffered past damages and will suffer future damages, both compensatory and general, including, but not limited to, front and back pay; loss of benefits; loss of accumulated sick pay; loss of retirement accounts; lost earnings on retirement funds; vacation time, compensatory time, and paid time off; negative tax consequences (in the event of a lump sum award), including related accountant fees; attorney's fees; emotional distress; mental, psychological and physical harm; loss of income; loss of enjoyment of life; for which defendants are liable in compensatory, punitive, exemplary, legal, equitable, and all other damages that this Court deems necessary and proper.

607. When the Defendants' behavior reaches a sufficient threshold, punitive damages are recoverable in § 1983 cases. *Smith v. Wade*, 461 U.S. 30 (1983)

608. Because Defendants' actions were intentional and willful, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

609. Because Defendant's actions involved reckless or callous indifference to the Plaintiffs' federally protected rights, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them,

individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

610. Because Defendant's actions were motivated by evil motive or intent, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

611. Plaintiffs seek recovery of attorney's fees under the Civil Rights Attorney's Fees Awards Act of 1976 and 42 U.S.C. § 1988, and under any other provision of law or basis.

612. Plaintiffs seek recovery of all court costs and out-of-pocket litigation expenses, including but not limited to expert fees, and legal interest on any amount of damages awarded

## XXVI. Jury Trial Demand

613. Plaintiffs are entitled to, and hereby demand, a trial by jury on all issues of fact herein.

WHEREFORE, Plaintiffs pray that Defendants be served with a copy of this Complaint and be duly cited to appear and answer same, and after due proceedings are had, there be judgment herein against the Defendants awarding Plaintiffs all damages claimed herein, plus legal interest, taxable costs, expert fees, and attorney's fees, and all other relief determined to be just and equitable by this Court.

Respectfully submitted,

Dated: August 23, 2023

**SCHEXNAYDRE LAW FIRM**

BY: /s/ *David J. Schexnaydre*
DAVID J. SCHEXNAYDRE, T.A., *Pro Hac Vice Pending*
Louisiana Bar Roll #: 21073
Mississippi Bar Roll #: 102557
Texas Bar Roll #24076142
2895 Highway 190 • Suite 212
Mandeville, Louisiana 70471
Telephone: (985) 292-2020
Fax: (985) 235-1089
david@schexnaydre.com

**JONCUS LAW P.C.**

BY: s/ Stephen J. Joncus
**Stephen J. Joncus**, OSB No. 013072
13203 SE 172nd Ave Ste 166 #344
Happy Valley, Oregon 97086
Telephone: (971) 236-1200
Facsimile: (971) 244-7997
steve@joncus.net

*Attorneys for Plaintiffs*